

ONTARIO
SUPERIOR COURT OF JUSTICE

BETWEEN:	)	
	)	
Bonnie Bradley	)	
	)	Warren Whiteknight and Casey Dorey for
Plaintiff	)	the Plaintiff
	)	
– and –	)	
	)	
Dr. Richard Wang	)	Kosta Kalogiros and Erin Chesney for the
	)	Respondents
Clinic Management and Technical Services	)	
	)	
Respondents	)	
	)	
	)	
	)	HEARD: Nov. 21-24, Nov. 27-30, Dec. 1,
	)	4-8, 2023, Oct. 28-30, 2024

REASONS FOR DECISION

VALLEE J.

Introduction

- [1] The plaintiff, Bonnie Bradley, is a Registered Nurse and at the relevant time worked in the emergency department at Lakeridge Health, a hospital in Oshawa. Over the course of her 15 year career, she had administered many vaccinations to patients and had received many herself. Her employment required her to have current vaccinations. She made an appointment to see her family doctor, Dr. Brankston, to have a tetanus vaccination and also to discuss a thyroid issue. At that time, Dr. Brankston was supervising a first year family medicine resident, the defendant Dr. Richard Wang. On December 1, 2015, Ms. Bradley attended for her appointment. She saw Dr Wang. He requisitioned blood testing for the thyroid issue and administered the vaccination. Ms. Bradley states that she experienced pain like white hot lightening in her arm at the injection site. She looked at the needle and saw it was not in the correct place for an injection – it was too high and too anterior. In the weeks and months following the vaccination, the pain continued. Ms. Bradley experienced pain going up her arm and developed a significant shoulder condition which will be

explained below. She states that it has prevented her from working and has significantly impacted her life.

- [2] Ms. Bradley brings this action against Dr. Wang alleging negligence and claiming various heads of damages. The action against Clinic Management and Technical Services has been discontinued.

Issues to be determined

- [3] What was the applicable standard of care on December 1, 2015 for a doctor to administer a tetanus vaccination?
- [4] Did Dr. Wang breach the standard of care?
- [5] If so, on a balance of probabilities, did the breach cause Ms. Bradley's injury?
- [6] If so, how much should she be awarded for general damages?
- [7] What are Ms. Bradley's on-going needs; and what is the cost of them?
- [8] Has Ms. Bradley incurred a past income loss? If so, how much? Will she incur a future income loss? If so, how much?
- [9] Has she incurred a pension loss?
- [10] Should she have returned to some form of modified work?
- [11] What are Ms. Bradley's out of pocket expenses?
- [12] What is OHIP's subrogated interest?

What was the applicable standard of care on December 1, 2015 for a doctor to administer a tetanus vaccination?

- [13] The plaintiff called Dr. Ken Berger to provide expert opinion evidence on the standard of care. He was qualified as an expert in family medicine to give opinion evidence on the standard of care in 2015 for administering intra-muscular deltoid vaccinations including the risks, benefits and side effects and the indicia of properly and improperly administered intra-muscular vaccinations.
- [14] The defendant called Dr. Nikolina Mizdrak to similarly provide expert opinion evidence on the standard of care. She was qualified as an expert in family medicine to opine on the standard of care applicable to family physicians in 2015 regarding administering intramuscular deltoid injections, the side effects and or indicial, if any, of properly or improperly administered intramuscular vaccinations
- [15] Dr. Berger and Dr. Mizdrak agreed on the applicable standard of care. The injection site must be located, which they referred to as landmarking. In order to do this, a doctor would

locate the end of the acromium process (the bone at the end of the shoulder), then apply two finger widths horizontally underneath it, as a measure to determine the correct site, then locate the middle of the deltoid muscle. The doctor would mark the site, disinfect the area and then administer the vaccine perpendicular to the skin.

Did Dr. Wang breach the standard of care

Ms. Bradley's evidence

- [16] Ms. Bradley's evidence regarding the location on her arm where the vaccination was injected is different from Dr. Wang's evidence as to how he would typically landmark the location and administer an injection. Ms. Bradley testified that her nursing training included doing intramuscular injections. Her steps included assessing a patient for weight and muscle tone, drawing up the medication, land marking and cleaning the area and then putting the injection in the middle of the deltoid muscle. After that, she would chart the injection, which included recording the lot number, the position where it had been injected and the patient's tolerance.

- [17] Ms. Bradley stated that prior to December 1, 2015, as a Registered Nurse, she had administered thousands of vaccinations. She had also received a number of vaccinations. She stated that she did not have an adverse reaction to any of them.

- [18] Ms. Bradley stated that Dr. Brankston had been her family doctor since 2009. She stated that she attended at his office on Tuesday December 1, 2015 to obtain a vaccination for tetanus, diphtheria, and pertussis known as Adacel. (I will refer to it as the tetanus vaccination.) She also wanted to have her blood checked for a thyroid condition. Instead of seeing Dr. Brankston, Dr. Wang came into the room. He was a first-year medical resident who was working at Dr. Brankston's clinic under his supervision. Ms. Bradley stated that she was somewhat surprised to see him as she was expecting to see Dr. Brankston. Dr. Wang filled out the form for the bloodwork and left the room to draw the vaccine. Ms. Bradley sat down in the chair by the desk. He returned and cleansed her left arm. She stated that she looked away because she did not want to watch the needle go into her arm. When the injection went in, she immediately had significant pain which she described as a white blast of lightning. She did not jerk her arm but yelled out. She stated that she turned her head and could see the needle which was still in her arm. The needle was not where she thought it should be. She described it as "way too high up and way too close" to the front of her arm in comparison to the middle of her deltoid, which was where it should have been placed. She stated that she was 100% certain that the needle was in the wrong place because she saw it. She denied that the pain affected her perception of the location. She could not specifically remember the length of the needle. She pointed to the location on her arm where the needle went in. During her initial appointments with Dr. Brankston and Dr. Mason after December 1, 2015, Ms. Bradley showed them the location where the vaccination had been injected. (During their testimony, both doctors pointed to the location on their arms to illustrate what Ms. Bradley had told them.)

- [19] Ms. Bradley stated that she did not see how Dr. Wang carried out his landmarking to determine the injection site because she was looking away. Ms. Bradley stated that she had been taught how to landmark an injection site. This involved finding the end of the acromion, placing two fingers underneath it horizontally on the deltoid muscle and then injecting into the middle of the deltoid. Ms. Bradley stated that she did not say anything to Dr. Wang about the location of the injection. She was too concerned about the pain. She stated that she tried to tell him three times about the significant pain but he did not seem to pay attention. She stated that she yelled out, stated “that really hurts” or “it hurt a lot” and “you’re not understanding me.” He did say, “Ya tetanus hurts” and then left the office. She did not say anything to Dr. Wang about the location of the needle nor to the receptionist on the way out of the office. During her testimony, Ms. Bradley pointed to the spot on her shoulder where the injection was given. Ms. Bradley stated that she had never felt pain from her previous injections. She had never had a patient react like this to an injection that she had administered.
- [20] I will set out here Ms. Bradley’s evidence of what occurred up to December 9, 2015 because the defendant states that her actions were inconsistent with the pain she describes, and partly to maintain the chronology.
- [21] Ms. Bradley stated that after she received the injection, she went to the lab to have blood drawn. She then drove for 30 minutes to have lunch with a friend who was in a bad emotional situation because she was going through a divorce. Ms. Bradley stated that part way through the lunch, she went to the bathroom and was crying because of the pain.
- [22] Ms. Bradley stated that she then went home and was feeling “pretty rotten”. Her husband, Grant, was at work that day. She laid on the couch and cried. When Grant came home, she told him that the vaccine had been injected into the wrong place and that she was having severe pain.

December 7, 2015

- [23] Over the next three days (Wednesday December 2 to Friday December 4, 2015), Ms. Bradley did not call Dr. Brankston’s office. She stated that she hoped the pain would go away but it was immense. Ms. Bradley stated that she called Dr. Brankston’s office on Monday December 7, 2015 and made an appointment for the next day. She went to work on December 7, 2015 and was able to manage because she was working in only the medical observation zone.

December 8, 2015

- [24] Ms. Bradley stated that she went to her appointment with Dr. Brankston on December 8. She told him that she was very concerned that there had been some nerve damage to her left arm because the injection had been improperly landmarked. She was having severe pain and had trouble using her arm at work. Dr. Brankston noted that Ms. Bradley’s pain had improved 50%. He prescribed Lyrica. She went to work after the appointment. At

1:08 p.m., she called Dr. Brankston and requested stronger pain medication, Dilaudid. He faxed a prescription for it.

- [25] Dr. Brankston stated Ms. Bradley reported a burning-like sensation. He believed it had a neuropathic origin. She was experiencing numbness in the distribution of the median nerve. He believed she likely had irritation of the nerve from the injection. He prescribed Lyrica which is commonly used for neuropathic pain.

December 9, 2015

- [26] Ms. Bradley stated that on the next day, December 9, she went to work but only lasted for 1.5 hours because she had too much pain. She could not work so she returned home. The pain was causing her to cry. She could hardly eat anything. At this point, her husband Grant began to look after all aspects of the home. Ms. Bradley did not return to work after this date.

Dr. Wang's evidence

- [27] Dr. Wang stated that during medical school, he had a day of training in how to do injections. He had a chance to practice on a dummy. A nurse instructor watched the students doing it. In his clerkship, he administered intramuscular deltoid injections. By the time he graduated from medical school he had done a few dozen injections. None of his supervisors stated that he was doing it incorrectly.
- [28] Dr. Wang described the process that he was trained to use for administering an intramuscular deltoid injection. He would locate the lateral tip of the acromion on bone, measure two finger widths down the arm from it, put his middle finger at the back of the arm and his thumb on the front of the deltoid to locate the center of the muscle and then give the injection in the center. Dr Wang stated that by December 1, 2015, he had completed five months of his family medicine residency. By that date, he administered a few dozen intramuscular deltoid injections. He was doing intramuscular deltoid injections independently. Every time he administered a vaccination, he did it the same way that he was trained.
- [29] With respect to his encounter with Ms. Bradley, Dr. Wang stated that he recalled the appointment but not all of the details. He recalled the injection but not the specifics. Nobody else was present when he gave Ms. Bradley the injection. He does not have a visual memory of where the needle was injected. Dr. Wang stated that he would have administered the injection the same way as he had been trained to do it. He stated that this is the only way that he does intramuscular deltoid injections. He stated that he would not administer an injection at the location shown by Ms. Bradley. He has never administered one there. Dr. Wang stated that the location identified by Ms. Bradley was significantly anterior to any place that he has ever inserted an intramuscular deltoid needle. It would not be a safe place to administer a vaccination. There would be a heightened risk of a bad outcome in contrast to administering in the midline of the deltoid. He did not recall Ms. Bradley's expressing concerns about pain three times during the encounter. A patient's

experiencing pain like lightning would not be a typical response to a vaccination administration. Dr. Wang stated that after he administered the vaccination he made a progress note in Ms. Bradley's chart and signed it at the bottom. After his signature, there is a notation, "Case Reviewed and agree with above – Edwin Brankston, preceptor". Dr. Wang stated that he had no recollection of discussing this matter with Dr. Brankston.

Credibility and Reliability of Evidence

- [30] Credibility is a significant issue in this case. In *Osmani v. Universal Structural Restorations Ltd.*, 2022 ONSC 6979 (CanLII) paras. 265 - 268, Justice Di Luca described the analysis for making credibility findings:

As has been often stated, there is no magic formula to be applied in determining whether a witness is telling the truth. There are many factors that may be relevant in determining credibility. Some of the key factors include: whether the witness' evidence is internally consistent, whether it is externally consistent with evidence from other witnesses or exhibits, whether the witness has a bias or motive to give evidence that is more favourable to one side or the other, whether inconsistencies in the evidence are about important or minor matters, what explanations are given for any inconsistencies, and whether the inconsistencies suggest that the witness is lying.

Credibility is to be assessed based on the totality of the evidence before me. The court is not to engage in a piecemeal determination of a particular witness' evidence standing alone without context and without reference to the balance of the evidence. That said, in conducting a credibility analysis, I am permitted to accept some, none or all of a witness' evidence.

Ultimately, the assessment of credibility is a task guided by common sense and reason. In this regard, I am guided by the following oft-quoted passage from *Faryna v. Chorny*, 1951 CanLII 252 (BC CA), [1951] B.C.J. No. 152, [1951] 4 W.W.R. (N.S.) 171, [1952] 2 D.L.R. 354 (C.A.) at para. 11:

The credibility of interested witnesses, particularly in cases of conflict of evidence, cannot be gauged solely by the test of whether the personal demeanour of the particular witness carried conviction of the truth. The test must reasonably subject his story to an examination of its consistency with the probabilities that surround the currently existing conditions. In short, the real test of the truth of the story of a witness in such a case must be its harmony with the preponderance of the probabilities which a practical and informed person would readily recognize as reasonable in that place and in those

conditions. Only thus can a Court satisfactorily appraise the testimony of quick-minded, experienced and confident witnesses, and of those shrewd persons adept in the half-lie and of long and successful experience in combining skilful exaggeration with partial suppression of the truth. Again, a witness may testify what he sincerely believes to be true, but he may be quite honestly mistaken. For a trial Judge to say "I believe him because I judge him to be telling the truth," is to come to a conclusion on consideration of only half the problem. In truth it may easily be self-direction of a dangerous kind. [Emphasis added.]

The Defendant's position

Dr. Wang's evidence

- [31] The defence states that a physician's testimony as to his usual practice is admissible as evidence of what he likely did on the day in question. In *Belknap v. Meakes*, (1989 CanLII 5268 BCCA, paras. 39 – 40), the court stated, "If a person can say of something he regularly does in his professional life and that he invariably does it in a certain way, that surely is evidence and possibly convincing evidence that he did it in that way on the day in question."
- [32] The defence states that Ms. Bradley's evidence and version of events is unreliable and incompatible with the surrounding events and circumstances. Viewed within the broader evidentiary context, Dr. Wang's version of events is more likely and ought to be preferred over Ms. Bradley's. The defence also states that the court must not infer a breach of the standard of care from Ms. Bradley's post-vaccine course.

Ms. Bradley's evidence

- [33] The Defendant states that Ms. Bradley's evidence should be rejected for several reasons. She had a pre-incident history of problems with her left shoulder from 2012 to 2014. She did not tell the medical expert physicians about this. This undermines the reliability of her experts' opinions and her own testimony. In addition, Ms. Bradley did not have the best line-of-sight as to where the needle was inserted. She looked away during the insertion. The process would have taken a few seconds. She turned her head back around when she felt pain and would have seen the needle just before it was being removed. She had only a fleeting opportunity to look at the needle from a distorted vantage point – looking down and over her left shoulder while reacting to and processing lightening-like pain. The short time during which she saw the needle, her poor viewpoint and the concurrent pain are enough to undermine her recollection of events.
- [34] The defendant alleges that the passage of time has impacted Ms. Bradley's memory. She changed her description of the needle insertion site over the years. On December 14, 2015, Ms. Bradley reported to Dr. Brankston that her "injection was given in the anterior deltoid

area”. On December 16, she reported to Dr. Mason that the vaccine had been injected anteriorly in the shoulder area; and she indicated directly over the anterior glenohumeral area. During their testimony, both Dr. Brankston and Dr. Mason pointed to these locations on their own arms and, in both instances, they pointed to areas that were even more anterior to where Ms. Bradley had pointed. During her examinations for discovery, Ms. Bradley indicated that Dr. Wang administered the vaccine 1.5 cm down, and 4 cm anteriorly from the lateral aspect of the deltoid. Then, at Ms. Bradley’s in-person assessment with Dr. Smith in 2022, Ms. Bradley identified yet another location being 5 cm distal relative to the lateral edge of the acromion, and 3 cm anterior relative to the midline of the deltoid.

[35] The defence states that Ms. Bradley’s post vaccination conduct is not consistent with the lightening-like pain that she experienced because:

- a. She did not raise any concerns with Dr. Wang or Dr. Brankston on the day of the vaccination.
- b. She did not document the injection location while at the clinic or at home.
- c. She did not take a photo of the injection location while the needle mark would have been visible.
- d. After the injection, she walked to the clinic basement to have her blood drawn. Then she drove 30 minutes to meet a friend, Carrie Morden, for lunch but did not mention the incident.
- e. When she and her husband were at home, she told him that she had experienced excruciating pain going down her arm but she did not seek medical attention. Her husband did not drive her to the hospital.
- f. She stated that she spent three days crying on the sofa; however, she contacted Dr. Brankston’s office on December 4, 2015 to follow up on the results of her blood work but she did not report the pain in her arm.
- g. Despite the terrible pain, Ms. Bradley returned to work and completed a full 12 hour shift.
- h. Ms. Bradley also worked on December 8, 2015 and remained on standby on December 9, 2015. Only on the third day of work she had difficulties and had to return home.
- i. On December 8, 2015, Ms. Bradley saw Dr. Brankston and did not mention the misplaced vaccination. There is no reference to it in his notes. Dr. Brankston stated that if he did not make a note of something, it was because Ms. Bradley did not tell him about it or he did not think that the detail was relevant.

[36] The defence states that Ms. Bradley was an unreliable historian when interacting with her own physicians and experts. For example:

Ms. Bradley testified in examination-in-chief that she missed 8-10 weeks of work in 2010 due to a back-strain, possibly missed two days in 2013 on account of a further back strain having been injured trying to restrain a psychotic patient, and did not miss any work at all in respect of a “Strep A” exposure in 2014. Yet, in cross-examination (and as borne out in the contemporaneous notes), Ms. Bradley admitted that with respect to her back in 2013, she actually missed over 15 weeks of regular work in 2013 on account of back problems. She further admitted that with respect to the “Strep A” exposure incident in 2014, she not only missed around two weeks from work, but she did not return to work notwithstanding that she was advised she did not have a Strep A exposure and did not need to take antibiotics. Notably, the healthcare provider seeing Ms. Bradley at the time described Ms. Bradley as a “poor historian” and queried in the diagnosis/plan portion of the consult note: “Strep exposure / absenteeism?” When testifying to the general state of her health, Ms. Bradley omitted or “forgot” a number of pertinent health issues, such as: recurring left shoulder issues (in respect of which she reported pain issues in 2012, 2013, and 2014, and in respect of which she also missed work), recurring chronic pain and myalgias (a result of which was Dr. Brankston suspecting she had fibromyalgia), and a recurring fatigue and sleep issues.

She did not tell her experts about her “chronic back pain”, left shoulder issues, fibromyalgia/chronic myalgias, fatigue or sleep disturbances. In the rare instances that she did touch on the topic of her back or shoulder, her history was inaccurate or incomplete.

Ms. Bradley never disclosed or produced to the Defendant clearly relevant information regarding her day-to-day abilities, such as her creation and operation of a Facebook group documenting her knitting of 100’s of lap-blankets to be donated (which she updated routinely) or her creation of intricate egg-shell carvings or knitted dolls for sale.

Analysis

Ms. Bradley’s sight line and description of injection location

- [37] I do not accept the defendant’s assertion that Ms. Bradley did not have the best line-of-sight when she was looking at the needle. She did not have to look over her shoulder. She only had to turn her head to the left and look down at her arm. She had only a short time to see the needle but it was enough for her to see the injection site. The location of the injection that she described to the various physicians may not have been completely similar; however, none of them were close to the mid-point of the deltoid muscle where the vaccination should have been injected.

Post Vaccination Conduct

- [38] Regarding her post vaccination conduct, I accept her evidence that she did raise a concern with Dr. Wang on the day of the vaccination. She stated that she told him three times that it really hurt. The fact that she did not document the injection location while at the clinic or at home does not affect her credibility. At that time, she likely did not envision the pain would continue nor did she likely contemplate commencing a lawsuit. Having pain in her arm would not have prevented her from walking to the clinic's basement to have her blood drawn. Driving 30 minutes to meet a friend for lunch is a bit unusual, given the amount of pain that she reported; however, it is not impossible. Ms. Morden was upset and focused on her own difficulties with her divorce. The fact that she did not recall Ms. Bradley's condition at the time is not surprising.
- [39] Ms. Bradley did not mention her pain when she called Dr. Brankston's office on December 4, 2015; however, she was calling only to make an appointment. Ms. Bradley went to work on December 7, 2015 and was able to manage because she was working in only the medical observation zone. This did not require her to do any strenuous activities.
- [40] Ms. Bradley states that she went to her appointment with Dr. Brankston on December 8, 2015. She told him that she was very concerned that there had been some nerve damage to her left arm because the injection had been improperly landmarked. She was having severe pain and had trouble using her arm at work. Dr. Brankston noted that Ms. Bradley's pain had improved 50%. He prescribed Lyrica. She went to work after the appointment; however, at 1:08 p.m., she called Dr. Brankston and requested stronger pain medication, Dilaudid. He faxed a prescription for it.
- [41] Ms. Bradley testified that on December 9, 2015, she went to work but only lasted for 1.5 hours because she had too much pain. Dr. Brankston called Dr. Mason and asked him to see Ms. Bradley to do nerve conduction studies to look for abnormalities consistent with her deficit. In the referral note, Dr. Brankston stated that she had a possible left brachial plexus secondary in some manner to the recent Adacel injection.
- [42] People have different ideas about when they should see a doctor for an issue. One person might think that seeing a doctor immediately after onset of symptoms is appropriate. Another might think that calling for an appointment four days later, as Ms. Bradley did, and seeing the doctor seven days later is appropriate because the issue might resolve on its own. None of this conduct is inconsistent with the pain that Ms. Bradley described in her testimony.
- [43] In 2022, Ms. Bradley undertook a charity project to crochet granny squares and sew them together to make lap blankets. She put out a request for other people to make granny squares and send them to her. This was very successful. Crocheting granny squares, creating small blankets and doing other crafts was not inconsistent with Ms. Bradley's pain and disability. She could do this at her own pace. She was likely using her dominant right hand.

Ms. Bradley's prior medical history

- [44] Ms. Bradley described the work of an ED nurse. It is strenuous and involves considerable lifting and carrying heavy items. The fact that she had back problems prior to December 1, 2015 is not surprising. In 2010, Ms. Bradley hurt her back lifting a patient and either herniated or slipped a disc. Dr. Brankston prescribed a lumbar back brace. She missed 8 to 10 weeks from work and then returned to modified duties. At the end of two months, she returned to her regular position. She had no residual issues. She had a more tender back and wore a back brace when she had a heavy patient load.
- [45] Ms. Bradley testified that on April 14, 2012, she attended at the Lakeridge Health ED in the morning. She had woken up with chest pain and a sore shoulder. She had shortness of breath. Her diagnosis was "CWP/likely MSK" which she explained meant chest wall pain likely musculo-skeletal. She was discharged three hours later. Dr. Brankston stated that Ms. Bradley went to the hospital because these symptoms can be a cardiac issue.
- [46] On April 23, 2012, Ms. Bradley saw Dr. Brankston. She reported that she had awakened with acute back pain at the bra line radiating into the left shoulder and arm. A chiropractic adjustment had settled her chest pain. She still had back pain and was unable to roll over in bed. She could not turn her head. Dr. Brankston noted that she had a "tender lower c-spine and left paraspinal area at level of left scapula". He ordered a bone scan and an MRI of the thoracic and c-spine. Ms. Bradley stated that she has a rib that slips out sometimes. It causes pain to radiate to her shoulder. During this event, she took some time off work.
- [47] Ms. Bradley saw Dr. Brankston again on April 26, 2012. In his note he stated that all of the tests that he had ordered were negative except for the bone scan which was pending. She was off work until May 6, 2012. She saw Dr. Brankston again on May 14, 2012. He noted that her bone scan was negative and suggested that she see a chiropractor. Ms. Bradley testified that she did not have any shoulder issues that affected her function in 2012. She stated that this issue has not been a problem in years.
- [48] On February 7, 2013, Ms. Bradley saw Dr. Brankston. His note states that Ms. Bradley was continuing to experience recurrent fatigue and recurrent aches and pains – even myalgias (muscle pain) during the night. His assessment was "likely fibromyalgia".
- [49] In April 2013, there was a period when Ms. Bradley was either off work or on modified duties for 15 weeks for a back injury. She went to an urgent care/walk-in clinic on April 22, 2013. The note states that she had acute back pain while she was at work. It had persisted for 10 days. It was in the middle of her low back and was radiating into her thighs to her knees which was not improving. She had been down on both of her legs. She could not straighten up or bend over. An x-ray of her lumbar spine was ordered to rule out a compression fracture.
- [50] On May 2, 2013 Ms. Bradley attended at Dr. Brankston's office. She reported that her back pain was much improved. The x-rays showed no evidence of a compression fracture. Ms.

Bradley was off work for 3.5 months until June. She returned to modified work duties on July 8. She did not recall being off work for this event that occurred 11 years ago.

- [51] On August 8, 2013, Ms. Bradley had a sore left posterior shoulder. The chiropractor noted that she may have jammed her lower back while catching a falling senior.
- [52] In October 2013, while at work, Ms. Bradley was attacked by a patient who was in acute psychosis. Her back was strained as a result of this. She saw Dr. Brankston on November 19, 2013. He noted that she had re-injured her back while trying to restrain a psychotic patient in the ER. She rested for five days with improvement and went back to work. She was then “whacked with a large door” and had another five days off. She went back to work with a back brace and had a lot of difficulty with back pain. Ms. Bradley stated that she made a WSIB claim. Dr. Brankston signed a WSIB form that day which stated that she was unable to return to work because of workplace injury and that there should be no work modifications at this time. He predicted that she would be off work from 8 to 14 days. Ms. Bradley’s recollection of this event that occurred 11 years ago was that perhaps she missed two days of work.
- [53] Ms. Bradley saw Dr. Brankston on November 27, 2013. His note states Ms. Bradley reported that her back pain had virtually settled. She was left with sacral pain all the time. Her back pain was intermittent, and recurrently chronic and associated with chronic stiffness.
- [54] On May 13, 2014, Ms. Bradley saw a chiropractor. His note state that she had symptoms for four weeks, which he described as left levator scapula strain.
- [55] On July 12, 2014, the chiropractor noted that Ms. Bradley had the same pain but worse in her left shoulder area and mentioned left levator tendinitis. Ms. Bradley described this as pain in her back radiating to her shoulder.
- [56] On October 28, 2015, Ms. Bradley attended at the urgent care/walk in clinic. She reported that she had injured her back at work doing patient care for a person who weighed over 500 pounds. The note states that she felt she could do full duties at work the next day.
- [57] Between 2012 and 2015, most of Ms. Bradley’s issues were back pain. In 2014, her chiropractor described her pain as left levator scapula¹ pain and left levator tendinitis. This is different from the pain she experienced after the vaccination.
- [58] Ms. Bradley’s recollection at trial in 2023 of the time she missed from work was different from what the medical records show; however, these events occurred in 2012 and 2014. The fact that she did not recall accurately the time she missed from work in periods over ten years prior to trial does not persuade me that she is an unreliable witness.

¹ Shoulder blade

Prior history is unrelated

- [59] Ms. Bradley’s prior medical history concerns back pain, sometimes radiating to her shoulder and shoulder pain in the area of her shoulder blade. Ms. Bradley saw the expert doctors in 2019 to 2023 about her shoulder condition after the vaccination. It was different from anything that she had experienced prior to the vaccination. There is no suggestion that her back pain was related to her post-vaccination shoulder issue. The fact that she did not mention dated back problems and a different type of shoulder problem to the expert doctors does not diminish her credibility or reliability nor does it diminish the reliability of those reports.

Conclusion

- [60] Understandably, Dr. Wang had no memory of the actual injection. Ms. Bradley testified that when she felt the pain and turned her head to look at her arm, she saw the needle still in her arm at a location that was higher up and more anterior than where it should have been – in the middle of her deltoid muscle. Ms. Bradley’s evidence is the best evidence regarding the incident. For the above-noted reasons, I accept Ms. Bradley’s evidence of what she saw. I find on a balance of probabilities that Dr. Wang breached the standard of care by failing to landmark properly and by administering the vaccination in the wrong location. It was not administered in the middle of the deltoid.

Did the breach of the standard of care cause Ms. Bradley’s injury?**Applicable Law**

- [61] The defendant states that to prove causation, the Plaintiff must prove that “but for” the Defendant’s breach of the standard of care, the injury would not have occurred. The Supreme Court of Canada explained the application of the “but for” test in detail in para. 8 of *Clements v Clements*, 2012 SCC 32:

The test for showing causation is the “but for” test. The plaintiff must show on a balance of probabilities that “but for” the defendant’s negligent act, the injury would not have occurred. Inherent in the phrase “but for” is the requirement that the defendant’s negligence was necessary to bring about the injury – in other words that the injury would not have occurred without the defendant’s negligence. This is a factual inquiry. If the plaintiff does not establish this on a balance of probabilities, having regard to all the evidence, her action against the defendant fails.

- [62] The Defendant states Ms. Bradley must demonstrate what would have happened had there not been a breach and how that would have impacted her ultimate outcome. In para. 134 of *Coffey v. Cyriac*, 2020 ONSC 6411, the court stated,

The mere possibility of a causal connection between the alleged negligence and the damage caused is insufficient. It is not enough

for the plaintiff to speculate, or to ask the trier of fact to speculate, about what could have happened but for the physician's actions. The plaintiff must lead specific evidence to establish on a balance of probabilities that but for the physician's alleged negligence a better or different outcome would have resulted. As the Ontario Court of Appeal stated in *Rothwell v. Raes*: "the onus is not met simply by demonstrating that there is the possibility of some causal connection."

[63] The plaintiff states that while the legal or ultimate burden remains with the plaintiff, positive or scientific proof of causation is not required. There is no need for scientific evidence of the precise contribution that the defendant's negligence made to the injury. (*Clements v. Clements* paras. 9, and 11)

[64] Evidence connecting the breach of the duty to the injury suffered may permit an inference that the defendant's negligence probably caused the loss. (*Ediger v. Johnston*, 2013 SCC 18 (CanLII) paras. 36 and 39)

[65] Where "but for" causation is established by inference only, it is open to the defendant to argue or call evidence that the accident would have happened without the defendant's negligence, for example that the negligence was not a necessary cause of the injury, which was in any event, inevitable. In para. 330 of *Snell v. Farrell*, 1990 CanLII 70 (SCC) paras. 83 – 85 the court stated,

The legal or ultimate burden remains with the plaintiff, but in the absence of evidence to the contrary adduced by the defendant, an inference of causation may be drawn although positive or scientific proof of causation has not been adduced.

[66] The causation inquiry is essentially a factual one based on the entirety of the evidence and made with a view to determining whether the plaintiff has established causation on a balance of probabilities and not a scientific certainty. The trier of fact may rely on inferences that are reasonably available on the evidence. (*Ediger v. Johnston*, 2013 SCC 18, para. 36)

[67] It is not essential in every case that medical experts provide a firm opinion supporting the plaintiff's theory of causation. Medical experts ordinarily determine causation in terms of certainties whereas a lesser standard is demanded by the law. Accordingly, legal causation may still exist where a medical expert has declined to draw a causal connection between the defendant's negligence and the plaintiff's injury. The judge or jury's power to draw an inference is not impaired by the failure of any medical witness to testify that a defendant's breach was in fact the cause, or by lack of medical unanimity. The matter does not turn on the use of a particular form or words used by physicians in giving their testimony. The judge or jury, not the medical witnesses, determine the issue of legal causation. (*Snell v. Farrell*, 1990 CanLII 70 (SCC paras. 34-36; *Fisher v. Atack*, [2008] O.J. No. 4481 (C.A.) para. 54.

Ms. Bradley's appointments with Dr. Brankston and Dr. Mason after the incident and their documents

- [68] The type of pain that Ms. Bradley experienced as well as its onset are important. Dr. Brankston stated that on December 9, 2015, he called Dr. Mason, a physiatrist (specialist in physical medicine and rehabilitation) and asked him to see Ms. Bradley in a timely way to do nerve conduction studies to look for abnormalities consistent with her deficit. He did not specifically mention the injection site. In the referral note, Dr. Brankston stated that she had a possible left brachial plexus secondary in some manner to the recent Adacel injection.

December 10, 2015

- [69] Ms. Bradley attended Dr. Mason on December 10, 2015. She filled out a questionnaire that inquired about health conditions, none of which she had. Dr. Mason did nerve conduction studies that showed there was no nerve damage. At that time, he did not make any treatment recommendations. Ms. Bradley stated that her pain was not decreasing. Her left arm was very weak. She was having trouble moving it around. She could not lift it over her head. Ms. Bradley stated that initially, the doctors thought that she was developing a frozen shoulder.
- [70] Dr. Mason testified that Ms. Bradley indicated to him that she had received the injection over the left glenohumeral joint. She reported that she had pain at the time of the injection which was quite strong and increased in severity. It disturbed her sleep. She had difficulty moving her left arm. She had tried to return to work but could not do the job. The pain had spread around her shoulder blade. She had tingling in her thumb and index finger. The pain had improved a bit but she still could not raise her arm properly. Dr. Mason testified that he tested her balance by having her walk. She could lift her arm to a certain point but then the pain became worse. When he supported the weight of her arm, she could move it throughout the range of motion. She had tenderness over the biceps tendon and glenohumeral joint. He performed nerve conduction tests. They were normal.
- [71] Dr. Mason testified that he wrote a consultation report to Dr. Brankston dated December 16, 2015, in which he stated that Ms. Bradley did not have any nerve dysfunction. She had a significant inflammatory response in the rotator cuff, particularly in the biceps tendon and the supraspinatus tendon and associated entrapment.
- [72] In the report, he states,
- I am form [sic] the patient that I do not think she has had significant brachial plexus or peripheral nerve injury, but that she has had significant inflammatory response in the rotator cuff, particularly the biceps tendon and the supraspinatus tendon and has associated entrapment. One suspects that the rotator cuff received the injection, and in fact some of the injection may have become intra- articular,

if the injection was into the biceps tendon sheath. If so, the patient may have developed a component of joint synovitis as well.

- [73] Dr. Mason testified that he believed this because of Ms. Bradley's description of where the injection occurred, the onset of symptoms immediately afterwards and where the pain was located. Her rotator cuff and joint had become inflamed. He stated that his initial recommendation, which was unusual for him, was that her arm be supported by a sling. She was uncomfortable. He recommended that she take it out to do range of motion exercises such as using her right arm to push her left arm up and outward to the side. She could do this without activating her sore muscles.

December 14, 2015

- [74] Ms. Bradley saw Dr. Brankston again on December 14, 2015. He noted that she was still having a lot of pain and had some weakness in her left arm. He noted that she maintained that the injection was given in the anterior deltoid area. Dr. Brankston testified that Ms. Bradley told him her injection was given in the anterior deltoid area. He stated that she had mentioned this at the previous visit but he had not documented it. He stated that this was important because the anterior deltoid area is not the appropriate place to give an injection. He did not review this with Dr. Wang to get his version of events.

December 17, 2015

- [75] On December 17, 2015, Dr. Brankston completed a medical certificate for employment insurance sickness benefits.

January 7, 2016

- [76] Ms. Bradley stated that she saw Dr. Brankston on January 7, 2016. She reported that she was significantly improved regarding her left arm and shoulder pain. She was not taking pain medication. He noted that her left shoulder movement was improved. He stated, "she may have a partially frozen shoulder - therefore a course of physiotherapy would be in order." He recommended physiotherapy and a trial of a reduced dose of Lyrica then tapering it over two weeks to zero.

January 14, 2016

- [77] Dr. Mason testified that he saw Ms. Bradley again on January 14, 2016. Her pain had improved but she still had tenderness over the same areas. His consultation report to Dr. Brankston stated that she was "markedly tender over the left biceps tendon and the left supraspinatus". He told Ms. Bradley that a steroid injection could be done if she did not improve. In his consultation report, he stated,

Her pain has improved considerably...On examination she had full active and passive range of motion of the left shoulder but is markedly tender over the left biceps tendon and the left supraspinatus...It is reasonable to have a trial of the physiotherapy.

If she does not respond to this, and time, with further pain relief than [sic] corticosteroid injection could be considered.

The experts' opinions

Dr. Ruggles' opinion

- [78] Dr. David Ruggles was retained by the plaintiff and was qualified as an expert in physical medicine and rehabilitation, permitted to give an opinion evidence on the causation and severity of musculoskeletal and soft tissue injury, including injuries to the shoulder and involvement of surrounding structures, as well as myofascial pain, chronic pain, peripheral and central sensitization associated functional, vocational, and avocational impairments arising from these phenomena and the strategies for treatment of them.

Dr. Ruggles' evidence

- [79] Dr. Ruggles stated he understood that while Dr. Wang was performing a vaccination, Ms. Bradley experienced sudden and severe pain, which was unusual. She said the vaccination was given anterior and higher than it should have been. She had continued to have severe pain for weeks. The basis for his understanding was what she reported to him as well as the records provided including Dr. Mason's consultation report just two weeks after the vaccination.
- [80] Dr. Ruggles stated that he also reviewed Dr. Brankston's note dated December 8, 2015. Ms. Bradley had a very atypical pain response to the vaccination. He stated that most people would have a dull ache for a day or two after a vaccination. Most people do not have severe pain as Ms. Bradley reported. Dr. Brankston described the pain in significant detail.
- [81] According to Dr. Brankston, Ms. Bradley was having significant pain one week later including pain at night which was disturbing her sleep. He stated that this was more in keeping with an injection into a non-muscular structure. He stated that muscle is more resilient than other structures which is the reason why intramuscular injections are done. Muscle allows for better absorption and the most immune response. Tendons do not. The deltoid muscle is thickest below the humerus head. A vaccination has to be given at a location that is several finger widths below the acromion. He conceded that people do not always have text book anatomy. Some structures might be situated a bit differently. There is variability among people. They can have different sized bursas.

Importance of Dr. Mason's opinion

- [82] Dr. Ruggles stated that Dr. Mason's opinion was important. Ms. Bradley saw him two weeks after the incident. Dr. Mason stated,
- She has had significant inflammatory response in the rotator cuff, particularly the biceps tendon and the supraspinatus tendon and has associated entrapment. One suspects that the rotator cuff received

the injection, and in fact some of the injection may have become intra-articular, if the injection was into the biceps tendon sheath. If so, the patient may have developed a component of joint synovitis as well.

- [83] Dr. Mason's description of where the vaccine was actually administered would be more reliable than that of any physician who saw her years after the incident because he saw her shortly afterwards.
- [84] Dr. Ruggles stated that acute physical findings such as pain with palpation can be reliable to pinpoint the source of the pain before the pain has spread. If certain movements cause pain, that can be another indication of a symptomatic point. Dr. Ruggles stated that the most important factors to determine what actually occurred were the location of the needle, the temporal nature of the pain at the time of injection and the continuous severe ongoing pain with no interruption. Dr. Mason's examination at that time was reliable with respect to the musculoskeletal structures from which the pain originated. In Dr. Mason's initial note, he described the pain from Ms. Bradley's rotator cuff as a painful arc. She had tenderness over her left supraspinatus tendon.

Symptoms caused by administration of vaccine

- [85] Dr. Ruggles testified that Ms. Bradley symptoms were caused by the administration of a vaccination resulting in an injury. She had musculoskeletal injuries in her shoulder area and had developed adhesive capsulitis. She had undergone a prolonged course of treatment for it. She had secondary pain problems because of the significantly impaired movement and function of her left shoulder. He explained that adhesive capsulitis is the same thing as frozen shoulder. Adhesions develop because of the inflammation which causes pain. He stated that a person can have a lot of pain which usually resolves in a month or two but then can go into a stuck phase because of adhesions that are developing. He stated that there is no consistently effective treatment to speed up the resolution of the frozen phase. Usually, people have to wait 18 to 24 months for the frozen phase to resolve.
- [86] Dr. Ruggles stated that determining when the adhesive capsulitis started would be hard to do because Ms. Bradley's pain never went away. Ms. Bradley still had significant restricted movement three years later. Typically, adhesive capsulitis should have been better by then.
- [87] Dr. Ruggles stated that based on Ms. Bradley's description and his physical findings, her pain was not just from her shoulder joint. The activities that aggravated it suggested to him that there was also pain coming from her neck and muscles in her shoulder area. If joint problems persist over a year, it is inevitable that surrounding soft structures will become involved.
- [88] Dr. Ruggles stated that when he saw Ms. Bradley three years after the incident, she was still having restrictions with her range of movement. A typical one year of adhesive capsulitis had passed by. He thought that perhaps she had developed a permanent contracture of her shoulder joint which meant that the muscles had become shortened

because of being in a fixed position; therefore, he was pessimistic about any further improvement in her range of motion. She had altered biomechanics due to the persistent contracture.

[89] Dr. Ruggles stated that Ms. Bradley reported to him that her function had improved. She experienced exacerbations with heavier physical activities such as housekeeping and strenuous movements of her shoulder. She was doing the housekeeping in an altered way by pacing herself. Her husband was assisting her.

[90] Dr. Ruggles stated,

In my opinion, Ms. Bradley's description of her previous symptoms, the results of her subsequent investigations and her current symptoms and physical exam findings are entirely consistent with again an initial inflammatory synovitis/arthritis of the glenohumeral joint induced by incorrect delivery of the vaccination into/around the glenohumeral joint area which resulted in secondary and particularly severe case of adhesive capsulitis/frozen shoulder.

[91] After seeing Ms. Bradley on January 22, 2019, Dr. Ruggles opined that Ms. Bradley would not be able to return to an ED nursing position, which involved heavier and unrestricted use of her non-dominant left upper extremity. However, she might be able to do a light nursing position. He stated that he would defer to a vocational assessment regarding Ms. Bradley's prospects.

[92] Dr. Ruggles stated that he cited a few papers in the 2019 report to show his understanding of literature regarding potential causes of shoulder injuries from vaccinations. He described it as mixed and contradictory. He stated that he reviewed the articles to some extent but did not rely on them in forming his opinion². He stated that people have to be assessed on an individual basis to determine why they develop shoulder pain after vaccination. There can be multiple reasons for this type of shoulder pain. It could be an abnormal immune response or an improper administration. It depends on the person, symptoms and evolution of pain.

Dr. Berger's evidence

[93] As noted above, Dr. Berger was qualified to provide expert opinion on the indicia of properly and improperly administered intra-muscular vaccinations, among other things. His evidence was focused on the standard of care; however, regarding the indicia, he commented on the likelihood of a person's experiencing extreme pain immediately after the administration of a vaccination, as was Ms. Bradley's experience. He stated that it would be extremely unlikely – a rare occurrence.

² I am not setting out the content of these articles because both Dr. Ruggles and Dr. Smith stated that they were controversial and that they did not base their opinions on them.

Dr. Smith's opinion

- [94] Dr. Jason Smith was qualified as an expert in orthopedics to provide an opinion on the issue of causation including opining on the concept of SIRVA³, the likely cause of Ms. Bradley's post injection shoulder pain and on the question of whether the location of the injection can be determined from her clinical course and circumstances. He was also qualified to opine on the issue of her post vaccine functional abilities including her prognosis and her ability to resume working after December 1, 2015.
- [95] Dr. Smith stated that Ms. Bradley's difficulties are a direct result from the vaccination administration. She had an acute inflammatory reaction to it. Reactions like this are not common but do occur. Both improperly administered and properly administered vaccines can cause the same results. One cannot infer from Ms. Bradley's symptoms that the vaccination was administered improperly.
- [96] Dr. Smith stated that he was familiar with atypical shoulder pain and dysfunction from vaccinations. Patients came to his office after having received Covid vaccinations. He stated that shoulder pain and dysfunction was very uncommon considering the number of Covid vaccinations that had been administered. He assumed that these vaccinations had been administered properly because the patients had told him so.
- [97] Dr. Smith stated that he has seen vaccines that were administered correctly that caused intense inflammatory reactions similar to Ms. Bradley's. Some people will have an acute and severe inflammatory reaction that results in widespread shoulder dysfunction. He stated that it all comes down to anatomy which can be variable, the proximity of the structures and the functions of each relating to each other.
- [98] Dr. Smith stated that Ms. Bradley had an acute, large inflammatory reaction that resolved and was then followed by shoulder dysfunction. A person can have an acute inflammatory reaction after receiving a properly administered vaccination. This is because a number of structures such as the bicep and supraspinatus tendons and the capsule are located under the area where a vaccination would be administered. They are on top of each other.
- [99] Dr. Smith stated that a vaccination can cause an inflammatory response in these structures. If the deltoid is inflamed and pressure is applied to it, pain symptoms can be elicited. One cannot determine that the injection was given improperly based on Ms. Bradley's report of acute pain on administration and then persistent pain and tenderness in the bicep and

³ SIRVA is an acronym for a condition discussed in the articles, which means Shoulder Injuries Related to Vaccine. Dr. Smith stated that he conducted a literature review on shoulder injuries related to vaccine administration because Dr. Ruggles had referred to it and he was not familiar with it. He stated that it is a medico-legal term, not a diagnosis. In his 2022 report, he states that it identifies symptoms that have arisen from vaccinations. It was introduced in 2010 by the Vaccine Injury Compensation Program in the United States (which was a no fault system) and has been used in various medical contexts when discussing some of the adverse effects of vaccine administration. He stated that very little is known about the pathologic mechanisms related to symptoms. Little is known about what causes them. The literature indicates that more studies are required. Dr. Smith stated that the majority of his opinion is based on his review of Ms. Bradley's charts and his medical examination of her.

supraspinatous tendons. They could all have an inflammatory reaction from an improperly placed or properly placed injection. Vaccination material diffuses. If there is inflammation in one structure, it can cause inflammation in another.

- [100] Dr. Smith reviewed a magnetic resonance image (MRI) of Ms. Bradley's shoulder taken two months after the injection. He took a measurement and stated that the structures are within 1.86 cm of each other. He stated that the image looked fairly normal. He did not note any unusual presentations in Ms. Bradley's anatomy. Dr. Smith reviewed Ms. Bradley's 2012 bone and gallium scan. He stated that it showed a broadly inflammatory process in the lateral aspect of her left humeral head.
- [101] Dr. Smith stated that after the acute inflammatory response resolved, Ms. Bradley's capsule became inflamed. She went on to develop adhesive capsulitis, known as frozen shoulder and had a long chronic case of it. Dr. Smith stated that Ms. Bradley had pre-existing risk factors for this condition: being female, approximately 50 years of age and having a hypothyroid issue.
- [102] Dr. Smith performed a detailed examination of Ms. Bradley's shoulder and arm function when she attended in July 2022. He stated that she had good functional abilities. The range of motion in her left shoulder was only a few degrees different from her right shoulder. There was no muscle wasting. He noted that previously she had a lot of restrictions in her left shoulder as noted by Dr. Brankston and Dr. Mason; however, because of her good range of motion in 2022, the prognosis for her was good. Dr. Smith stated that Ms. Bradley would continue to improve.
- [103] With respect to return to work, Dr. Smith stated that Ms. Bradley could have returned to work in July 2022 with some restrictions. These would include lifting only a limited amount of weight, no overhead lifting and no pushing or pulling of more than 10 pounds. Dr. Smith also stated that Ms. Bradley could have returned to modified work such as a desk job sometime in 2016 when Dr. Mason noted that the inflammatory capsulitis was improving. He acknowledged that her treating physicians signed a number of forms stating that she could not return to work then; however, he disagreed with this.

Dr. Ruggles' response to Dr. Smith's opinion

- [104] Dr. Ruggles saw Ms. Bradley again on February 15, 2023 to clarify his opinion regarding causation, to assess her current ongoing symptoms and physical findings and to comment on Dr. Smith's independent medical evaluation report dated July 22, 2022. Dr. Ruggles noted that Ms. Bradley's range of motion was considerably better than it had been in 2019. She reported a full active range of motion for the past year but continued to have constant pain and predictable exacerbations of it with certain activities. In his report, he stated,

It is my opinion that Ms. Bradley's ongoing left shoulder pain and exacerbations of her left shoulder pain are due to a combination of her persistent altered biomechanics of her left shoulder and some degree of dynamic rotator cuff impingement as well as some degree

of peripheral and central sensitization caused by the particularly severe left shoulder pain that she experienced in the days, weeks and months after her vaccination was performed in December 2015, as well as her subsequent unusually prolonged course of secondary adhesive capsulitis... She continues to experience increased left shoulder pain beyond 60° of abduction and approximately 80° of forward flexion and has evidence of ongoing scapular dyskinesis on the left that is not present on the right, reflective of some degree of dynamic rotator cuff impingement and some ongoing irritability of her left shoulder joint... She also continues to experience increased sleep disturbance related to her persistent left shoulder pain and psychological symptoms including ongoing emotional distress...

- [105] With respect to peripheral and central sensitization, Dr. Ruggles explained that in the past, people who had chronic pain were thought to be malingerers. Now, there is a better understanding of how chronic pain evolves. There are well documented and accepted physical mechanisms and biological reasons that account for why people have chronic pain. Persistent signals of pain to the brain can cause the brain's perception and processing of pain to be altered permanently because it is now sensitized. (Dr. Ruggles' more fulsome explanation regarding sensitization is set out below.) Dr. Ruggles stated that given the length and time of Ms. Bradley's symptoms, her pain conditions arising from improper vaccination and her prognosis, her functional symptoms were unlikely to change. Her prognosis was poor.

Chest pain radiating to right shoulder

- [106] Dr. Ruggles asked Ms. Bradley about a reference that Dr. Smith made in his report about earlier shoulder pain. He stated that Ms. Bradley told him that she woke up with chest pain radiating to her shoulder and went to the ER. From the ER note dated April 12, 2012, Dr. Smith incorrectly interpreted CWP to mean chronic widespread pain. Dr. Ruggles stated that it means chest wall pain. Tests showed that it was not a cardiac event. Other causes were ruled out. The symptoms resolved.
- [107] Dr. Ruggles noted that Dr. Smith had opined that Ms. Bradley's imaging of her left shoulder following vaccination was consistent with significant pre-existing injuries that would have put her at significant risk of developing significant left shoulder pain and dysfunction as she did immediately following the 2015 vaccination. Dr. Ruggles disagreed that Ms. Bradley had any significant pre-existing issues. He commented that she did not report any and nothing was documented in the clinical notes and records. No investigations were carried out until after the vaccine. She had no previous treatment for left shoulder pain or issues. The MRI upon which Dr. Smith based his opinion does not disclose whether Ms. Bradley had pre-existing conditions. Most people over the age of 40 will have age-related degenerative changes of skeletal systems. If someone over 40 had no evidence of this on an MRI, that would be very unusual.

[108] Dr. Ruggles stated that his understanding of the injection's location was informed by Ms. Bradley's description. Dr. Brankston's notes and Dr. Mason's consult notes were also informed by Ms. Bradley's description. Dr. Ruggles stated that there were other factors as well: her description of the pain, the immediate onset, its persistence, its effect on her function and ability to sleep. He stated that it was not just about the location that she reported. Her symptoms correlated the location.

[109] Regarding causation, in his report dated February 25, 2023, Dr. Ruggles stated,

While I agree with Dr. Smith that an inflammatory response and secondary adhesive capsulitis is possible after proper administration of a vaccine due to the development of an abnormal immune response to the vaccine material/injectate itself, the onset and evolution of Ms. Bradley's left shoulder symptoms following her vaccination of December 1, 2015 would not be consistent with this...Ms. Bradley had an immediate severe pain while the vaccine was being administered which continued uninterrupted in the hours, days, weeks and months following administration of the vaccine. This would be much more consistent with an immediate/acute injury to some musculoskeletal structure in the left shoulder joint area caused by improper administration of a vaccine and a direct traumatic injury related to the needle and improper injection of a foreign material/irritant into a musculoskeletal structure that should not be injected with a vaccine. The immediate severe and uninterrupted severe pain that Ms. Bradley experienced in the hours, days and weeks following the vaccination (as is again clearly documented in her medical record) is not consistent with the milder muscular discomfort commonly associated with proper administration of a vaccine into the deltoid muscle and the possible development of an abnormal immune response that would cause a delayed inflammatory response that would typically take many day, if not weeks, to develop.

[110] Dr Ruggles concluded that Ms. Bradley's ongoing left shoulder symptoms and associated problems and functional limitations are a direct result of her vaccination of December 1, 2015 and more specifically improper administration of the vaccine rather than a delayed abnormal immune response to a properly administered vaccine.

[111] With respect to Dr. Smith's opinion that Ms. Bradley could have gone back to work in 2016, Dr. Ruggles stated that her condition was worse then and she was still having pain three years later in 2019. He stated he could not imagine her returning to work in 2016.

[112] Dr. Ruggles noted that Dr. Smith had opined that Ms. Bradley's imaging of her left shoulder following vaccination was consistent with pre-existing injuries that would have put her at significant risk of developing significant left shoulder pain and dysfunction as she did immediately following the 2015 vaccination. Dr. Ruggles stated that the MRI upon

which Dr. Smith based his opinion does not disclose whether Ms. Bradley had pre-existing conditions. Most people over the age of 40 will have age-related degenerative changes of skeletal systems. If someone over 40 had no evidence of this on an MRI, that would be very unusual.

The defendant's position

- [113] The Defendant states that Ms. Bradley has not proven that the breach is the “but for” cause of her shoulder/arm dysfunction. While the Plaintiff has led expert opinion evidence on what likely happened, she has not addressed what would have happened “but for” the breach. The Plaintiff has not led any opinion evidence that had the immunization been administered in the correct location, it would likely have resulted in a materially better outcome for Ms. Bradley.

Analysis

- [114] I question the foundation for Dr. Smith’s statement that Ms. Bradley had a pre-existing shoulder condition. In 2012, she had pain that radiated to her shoulder. It completely resolved. She had other episodes of shoulder blade pain that seem to have been job related. They resolved.
- [115] Tetanus vaccinations are common. Many people receive them without any issue. Ms. Bradley’s symptoms are not equally consistent with a properly placed vaccination. She had an immediate reaction to it – whereas an inflammatory response would typically take many days, if not weeks, to develop.
- [116] I must determine on a balance of probabilities whether Ms. Bradley’s shoulder symptoms were caused by the incorrectly administered vaccination.
- [117] Dr. Mason’s evidence is important. He is a physiatrist and was a treating physician. He initially saw Ms. Bradley on December 10, 2015, just nine days after the vaccination. After that appointment, he wrote a report to Dr. Brankston stating:
- She has had significant inflammatory response in the rotator cuff, particularly the biceps tendon and the supraspinatus tendon and has associated entrapment. One suspects that the rotator cuff received the injection, and in fact some of the injection may have become intra- articular, if the injection was into the bicep’s tendon sheath. If so, the patient may have developed a component of joint synovitis as well.
- [118] Dr. Mason stated that his suspicion was based on Ms. Bradley’s description of where the injection occurred, the onset of symptoms immediately afterwards and where the pain was located. Her rotator cuff and joint had become inflamed.
- [119] I give his evidence considerable weight because he is a specialist who saw her close in time to the event. He continued to see her until 2018.

- [120] The defendant states that the trier of fact should be extremely cautious and indeed reluctant to draw on common sense inferences to establish causation when expert medical evidence of causation had been led by the defendant. In *Moore v. Castlegar* (49 BCLR (3D) 100, par 11), the court stated, “I think in a case such as this where there is affirmative medical evidence leading to a medical conclusion, it is not open to the court to apply “the common sense reasoning urged in *Snell v. Farrell*...”
- [121] Dr. Ruggles was qualified as an expert in myofascial pain and chronic pain among other things. Dr. Smith was not qualified as a pain expert. Dr. Ruggles stated that an uncommon inflammatory reaction would typically take many days, if not weeks, to develop. Dr. Smith could not reach a conclusion as to whether the vaccination had been properly or improperly placed. An uncommon event is an outlier, a small possibility. Ms. Bradley’s symptoms developed immediately, not over days and weeks. The fact that Ms. Bradley had a number of tetanus vaccinations over the course of her career without any adverse reaction is significant.

Conclusion

- [122] Based on the evidence before me, including Ms. Bradley’s immediate onset of symptoms, I cannot conclude, on a balance of probabilities, that Ms. Bradley’s immediate significant inflammatory reaction to this vaccination was one of those uncommon, extremely unlikely, rare events. I find that the breach of the standard of care caused Ms. Bradley’s injury. But for the incorrect administration of the vaccine, Ms. Bradley would likely not have had the inflammatory reaction to it. She likely would have had no reaction, consistent with her history of receiving tetanus vaccinations.

General damages: How much should be awarded to Ms. Bradley for general damages?

- [123] Ms. Bradley requests that her general damages be assessed in the range of \$175,000 to \$225,000. The defendant states that \$40,000 is appropriate. The defendant states that Ms. Bradley’s injuries are not as severe as those sustained by the plaintiffs in the cases presented. Ms. Bradley’s continuously improving pain is limited to her left shoulder. Her physical condition has continued to improve significantly over time. As such, an award of \$40,000 in general damages is appropriate in the circumstances.

Applicable Law

- [124] In *Ratych v. Bloomer* (see, [1990] 1 S.C.R. 940, para. 71) the Supreme Court of Canada has stated:

...the general principles underlying our system of damages suggest that a plaintiff should receive full and fair compensation, calculated to place him or her in the same position as he or she would have been in had the tort not been committed, insofar as this can be achieved by a monetary award. This principle suggests that in calculating damages under the pecuniary heads, the measure of the damages should be the plaintiff’s actual loss.

- [125] In *Matthews Estate v. Hamilton Civic Hospitals*, 2009 Can LII 52312, paras. 172 – 173, the court stated, “...the court must attempt to restore the plaintiff as far as money can do so, to the position he or she would have been in before the tortious act, with all of its attendant risks and shortcomings and not to a better position... Therefore, the court must compare a plaintiff’s lot in life after the defendant’s tort with that which the plaintiff would probably have enjoyed but for the tort.”

Ms. Bradley’s pre-incident health

- [126] The defendant states that Ms. Bradley had a history of shoulder pain occurrences that must be taken into account. I disagree for the reasons set out above. Between 2012 and 2015, most of Ms. Bradley’s issues were back pain. In 2014, her chiropractor described her pain as left levator scapula⁴ pain and left levator tendinitis. None of Ms. Bradley’s previous shoulder pain issues were similar to the shoulder pain that she experienced as a result of the vaccination and that she continues to experience. They resolved prior to the incident.
- [127] Ms. Bradley stated that before the incident, her health was good. In the few years leading up to the incident, she had no psychological issues and no shoulder issues. She worked in the emergency department of Lakeridge Health as a nurse. She took medication for her thyroid as well as a sleep aid to help her sleep during the day because she worked shifts.

Ms. Bradley’s pre-incident recreation

- [128] Ms. Bradley stated that before the incident she and her husband liked to do a lot of outdoor activities. They hiked, walked, swam, biked and went camping and kayaking. They would go camping a couple of times each summer. With respect to hiking, they would drive to a location within a three hour radius of their home and go for a hike. They would take lunch and be gone for the entire day. They particularly loved kayaking. In the summer of 2015, they kayaked as often as they could, generally once or twice a week. She and her husband were social and went out to see friends.

Post Incident

- [129] Ms. Bradley continued to see Dr. Brankston up to September 14, 2016 when he retired. She saw Dr. Mason until August 2017. Dr. Bathe took over Dr. Brankston’s practice. Ms. Bradley saw him until 2023. Their evidence and documents during these periods are important.

⁴ Shoulder blade

February 17, 2016

- [130] Dr. Brankston ordered magnetic resonance imaging of Ms. Bradley's left shoulder. It was done on February 17, 2016. The report stated that there was mild degenerative change in the posterior lateral humeral head. No rotator cuff injury was seen.

February 25, 2016

- [131] Ms. Bradley stated that she saw Dr. Brankston on February 25, 2016. According to his note, she reported to him that she felt her left arm pain had essentially plateaued. Dr. Brankston noted that her left shoulder had full range of motion with passive movement. She had a lot of pain with active movement. He recommended that she restart Lyrica and that she see Dr. Mason again regarding a steroid injection. He also recommended that she have acupuncture. He completed a PRM form on which he stated that she was able to work with restrictions similar to those on the January 26, 2016 form.

March 15, 2016

- [132] On March 15, 2016, Dr. Mason sent a consultation report to Dr. Brankston. He stated that he had injected Depo-Medrol into her shoulder with only mild improvement in her pain and range of motion.
- [133] On the same date, Ms. Bradley saw Dr. Brankston. According to his note, she reported that the Depo-Medrol injection had provided no benefit at all. She was having a difficult time sleeping. Dr. Brankston filled out another PRM in which he stated that Ms. Bradley could not work with restrictions. Dr. Brankston stated that Dr. Mason's consultation report did not comment on her ability to return to work. He relied on Ms. Bradley's self-reporting. He did not do a focused physical examination.

April 14, 2015

- [134] On April 14, 2016, Dr. Mason prepared a consultation report for Dr. Brankston. He stated that since her last appointment with him on March 15, 2016, Ms. Bradley had a bone scan/gallium scan "which is consistent with inflammatory synovitis/arthritis in the left shoulder". He noted that she described some improvement in her pain but her range of motion had declined. His impression was that her pain had finally started to make some improvement, either because of the injection and medication changes or because the inflammatory phase of her "frozen shoulder syndrome" had started to subside.
- [135] On April 14, 2016, Dr. Brankston filled out an attending physician's statement form for the Bank of Montreal relating to mortgage disability insurance. He indicated that Ms. Bradley's physical impairment was "Class 5 – Severe limitation of functional capacity; incapable of minimal (sedentary) activity (75-100%)"

May 12, 2016

- [136] On May 12, 2016, Ms. Bradley saw Dr. Brankston. He completed another PMR form. He stated that she was still unable to work with restrictions. He also completed a form for Service Canada in which he stated that Ms. Bradley has having “complex regional pain syndrome L shoulder/arm”. He stated that the prognosis was “poor”.

June 9, 2016

- [137] Dr. Brankston completed another PMR form. He indicated that Ms. Bradley was unable to work with restrictions. On the same date, Dr. Mason prepared a consultation report for Dr. Brankston. He stated he discussed with Ms. Bradley various forms of injections including a suprascapular nerve block. She would have a shoulder x-ray.

June 14, 2016

- [138] Ms. Bradley saw Dr. Mason again. In his consultation report to Dr. Brankston, he stated that he had reviewed the x-rays which showed decreased bone density in the humeral head and proximal humerus but the joint surface was smooth and the joint space appears to be preserved. Dr. Mason stated that it probably represented “localized osteoporosis related to her CRPS/frozen shoulder”.

July 4, 2016

- [139] Ms. Bradley saw Dr. Brankston. She was using minimal Dilaudid and had stopped physiotherapy. She stated that Dr. Mason had advised her to stop it. She was having massage therapy and was seeing an osteopath. Dr. Brankston suggested acupuncture.

August 9, 2016

- [140] Dr. Mason prepared a consultation report to Dr. Brankston based on his re-assessment on that date. Ms. Bradley’s shoulder pain had become more tolerable but she had new neck pain. His impression was that she was having an episode of acute mechanical neck pain that would likely resolve within six to twelve weeks.

September 1, 2016

- [141] Dr. Mason saw Ms. Bradley. He noted that she still had significant discomfort. A repeat steroid injection or a nerve block and manipulation under anaesthesia were options she could consider. Dr. Mason stated that Ms. Bradley did not proceed with them.

September 14, 2016

- [142] Dr. Brankston last saw Ms. Bradley. He noted that she had been experiencing recurrent associated left trapezius spasm. He wrote a note for her to have acupuncture therapy.

- [143] Dr. Brankston subsequently retired. Dr. Stephen Bathe took over the practice. Ms. Bradley continued on with him.

October 24, 2016

- [144] Dr. Bathe completed a PRM form. He acknowledged that the form stated that if the patient was not able to perform “the regular duties of her job, we are able to provide modified work in most cases”. He stated on the form that Ms. Bradley had a “left arm disability with reduced mobility due to chronic frozen left shoulder and neuropathic left arm pain”. He noted that Ms. Bradley was unable to work with restrictions. On the same date, Dr. Bathe completed an Attending Physician’s Statement of Continuing Disability for Sun Life Financial. He stated that she had an inability to use her left arm and that she could not lift any weight. She was unable to return to work. He checked off the boxes stating that she had severe impairment regarding driving, climbing and lifting anything. She had no limitations for sitting and walking. Dr. Bathe stated that these were her subjective reported symptoms. Regarding driving, Dr. Bathe was not sufficiently concerned to review the Ministry of Transportation’s lists of impairments. Dr. Bathe acknowledged that he did not perform a functional capacity evaluation at this time.

January 12, 2017

- [145] Dr. Bathe saw Ms. Bradley. His note states that she had ongoing left shoulder/neck pain. She had seen Dr. Mason who recommended more time for improvement. He observed that she was in no apparent distress and she was holding her left arm close to her body. He did not do a functional assessment. On the same date, he completed an PRM form in which he stated that Ms. Bradley remained totally disabled from performing her regular duties.

February 9, 2017

- [146] Dr. Bathe completed an Attending Physician’s Statement of Continuing Disability for Sun Life in which he stated that her progress remained unchanged. He did not do a functional assessment. Ms. Bradley had gained some weight. Dr. Bathe stated that he counselled Ms. Bradley about trying some exercises and reducing calories. She could do modified exercises for her lower body.

March 14, 2017

- [147] Dr. Bathe completed a form for Johnson Inc. Ltd stating that there was a little improvement and she was still not capable of working.

May 2, 2017

- [148] Ms. Bradley saw Dr. Mason. In his consultation report to Dr. Bathe, he stated that she reported she was making improvement and her pain was less severe. She still had a decreased range of motion in her shoulder but she had improved from the previous assessment. He stated that she was finally making improvement. He did not suggest any new interventions.

June 8, 2017

- [149] Dr. Bathe completed a form for Johnson Inc. Ltd in which he stated that Ms. Bradley was slow in her improvement but making progress overall. Dr. Bathe also completed a form for Health Care of Ontario Pension Plan (HOOPP) in which he stated that Ms. Bradley was unable to work at any occupation because she was unable to use her left arm due to neuropathic pain.

August 7, 2017

- [150] Dr. Bathe completed an Attending Medical Practitioner's Assessment for HOOPP. It stated that she had chrono-neuropathic pain in her left shoulder. Her condition was unchanged.

August 17, 2017

- [151] Ms. Bradley stated that after the incident, she was not working so she and her husband could no longer afford the mortgage payments on their house in Courtice. Her husband was familiar with the town of Marmora so they purchased a house there. They moved on August 17, 2017. It is 3,500 ft.², has three floors and four bedrooms, which are all upstairs. It has four bathrooms, one of which is on the main floor and one which is in the basement. The laundry is also located in the basement. A physiotherapy room has been set up for her in the basement which her husband built. She stated that she uses it every day.

August 31, 2017

- [152] Ms. Bradley saw Dr. Mason. In his consultation report to Dr. Bathe, he stated, "Examination shows that active abduction of the left shoulder is still very impaired at 60 **degrees ADD SYMBOL** and forward elevation at 95°. Her passive glenohumeral range of motion had improved mildly. He did not suggest any new interventions.

January 23, 2018

- [153] Ms. Bradley stated that in 2018 she started to see some improvement in her range of motion which allowed her to do some activities. On January 23, 2018, Ms. Bradley saw Dr. Mason again. He testified that she was having physiotherapy to improve her range of motion. He wrote a report to Dr. Brankston stating, among other things, "The impression is that her rest pain has continued to improve. She has improved her active use of the left upper extremity and active range of motion of the left shoulder. However, passive glenohumeral range is still significantly reduced."

March 14, 2018

- [154] Dr. Bathe saw Ms. Bradley and completed a form for Johnson Inc. Ltd on which he stated there was a little improvement but she was still not capable of working. She had a class 4 physical impairment ("marked limitation – capable of minimal activity) and a class 2 psychiatric impairment (able to function in most stress situations and engage in most interpersonal relationships – slight limitation).

July 8, 2018

- [155] Dr. Bathe completed another HOOPP form. He stated that Ms. Bradley was not able to work at any occupation at all.

August 2, 2018

- [156] Dr. Mason saw Ms. Bradley. His impression was that her frozen shoulder was finally making improvement regarding resting pain and range of motion. He stated in his consultation report to Dr. Bathe that they had a discussion about possible return to work. He stated that she could not return to the usual duties of a registered nurse at that time. He had a discussion with Ms. Bradley about possible return to work, eventually.

August 7, 2018

- [157] Dr. Bathe saw Ms. Bradley and completed another form for HOOPP. On it, he stated that her condition remained unchanged. He estimated that Ms. Bradley could possibly return to work in a year perhaps based on what Dr. Mason had said. It was a conservative estimate. He did a physical examination of Ms. Bradley at that time. He stated that she was unable to work at any occupation.

March 25, 2019

- [158] Dr. Bathe saw Ms. Bradley and noted that she had very mild improvement overall. He completed another PRM form. It contained the same description of her impairments. Dr. Bathe did not do a physical examination of Ms. Bradley at the time.

May 4, 2020

- [159] Dr. Bathe saw Ms. Bradley. Ms. Bradley reported that her relationship with her husband was strained. They had attended counselling but there had been no benefit. Her husband will have treatment for depression.

June 5, 2020

- [160] Dr. Bath completed an Attending Medical Practitioner's Assessment for HOOPP. He stated that Ms. Bradley had a limited range of motion in her left shoulder and that she was not able to work at her own occupation full time or part time because she had a physical limitation that prevented her ability for ED nursing. She was currently able to work at any occupation full time or part time. Dr. Bathe testified that he meant Ms. Bradley could work as a Walmart greeter or in a job with voice activation.

September 29, 2020

- [161] HOOPP sent a letter to Ms. Bradley dated September 29, 2020 stating that she no longer qualified for free accrual from HOOPP after the fourth year of her health leave. This was because she did not meet their definition of being either totally disabled or permanently

disabled. It stated that she had three options: return to work for a HOOP employer and resume making contributions; appeal this decision to an Independent Medical Examiner or request termination options from HOOPP.

October 20, 2020

- [162] Dr. Bathe saw Ms. Bradley. He noted that she advised him that she had submitted the HOOPP forms but had been told that she either had to go back to work or have a HOOPP medical assessment. She had not regained full range of motion in her left shoulder and felt that she could not return to her 2015 duties.
- [163] Ms. Bradley stated that she stopped seeing Dr. Mason because there were no changes and there was nothing more that he could do.

August 14, 2021 Dr. Auguste

- [164] Dr. Auguste, an orthopaedic surgeon, carried out an assessment of Ms. Bradley for HOOPP and prepared dated August 14, 2021. This was for the purposes of determining whether she was totally disabled as defined by HOOPP. The definition was: “a condition causing physical or mental impairment which prevents a claimant from engaging in any employment for which the claimant is reasonably suited by virtue of the claimant’s education, training or experience”. Dr. Auguste determined that Ms. Bradley’s current impairments were likely permanent. She was not disabled from any work as a RN. She was unable to return to her position as a RN in Lakeridge Health’s ED. Dr. Auguste recommended a job change to a “more administrative, less physical and more supervisory related position in the field of nursing”. She deferred to a vocational assessor in this regard.

August 19, 2022

- [165] Dr. Bathe saw Ms. Bradley. He noted that she had some improvement in her range of motion in her left shoulder but pain remained with activities such as hanging laundry. It would get to 3/10 but never go away. She was doing physio at home. Dr. Bathe completed another form for Manulife in which he stated that Ms. Bradley was not able to complete the tasks of an ED nurse.

January 18, 2023

- [166] Dr Bathe stated his last contact with Ms. Bradley was on January 18, 2022. It was a phone call to review her blood work. His note shows that they discussed the risks and benefits of her medications.

Chronic pain - sensitization

- [167] Ms. Bradley’s limited use of her left arm and her pain is well documented by her physicians and her life care planner (as set out below). It prevents her from doing many of the activities that she previously enjoyed. She has chronic pain. Dr. Ruggles explained why people have

chronic pain when the initial injury or inflammatory response has resolved. He also explained the consequences of having an immobile joint. In his testimony, he stated,

[T]hose [central and peripheral sensitization] are just one of the various mechanisms, sort of mechanisms that can account for why people have chronic pain when the original sub-acute injury, or acute medical issue, or acute inflammatory response has resolved. So, as discussed a little bit earlier when people have persistent acute and then sub-acute pain, that ongoing nociception – those nociception are basically pain signals going into the central nervous system, and, you know, lead up to the brain, the cortex of the brain where pain is processed can have – can induce changes, like hardwired changes, permanent changes to the peripheral and central nervous system. So those changes occur both at the peripheral nerve level, at the spinal cord level, and at the cortical level. And, again, these processes of peripheral central – peripheral and centralization and kind of cortical neuroplastic changes that occur with persistent severe pain are quite well documented and accepted. I mean, it's been known for decades. We used to feel that chronic pain patients were malingerers. That their fracture, or their soft tissue injury is resolved, why are still having pain? They must be looking for something malingerer, secondary gain. But again, the field of chronic pain in the last 30 years has evolved considerably. We now have a much better understanding of the physiologic and biologic mechanisms of chronic pain, and that also that it's even more complicated than that. People – there are psychological and social issues that affect people's pain experience as well, their pain perception and how their level of disability related to their pain. But again, there are definite biologic mechanisms that explain chronic pain, and that's just one mechanism. The other mechanism that I felt was responsible for Ms. Bradley's ongoing pain is that when people have persistent pain in a certain area that if the movement and function of that area is significantly altered for a period of time, other structures become secondarily involved. So the shoulder joint is normally a very mobile joint. When it's immobile for weeks, months, and about a year or two, it's almost inevitable that the surrounding soft tissue structures, which are intimately connected with the joint, the shoulder girdle muscles and the neck muscles become secondarily involved. If they're not moving properly, they shorten. They become weak. They become deconditioned. And then people develop persistent myocardial pain, which is persistent localized pain.

Cases regarding general damages presented by the plaintiff

- [168] In *Frampton v. Fransons*, 2010 ONSC 675 the plaintiff received an electric shock, fell from 10 – 15', suffered a very serious crushing fracture of his right shoulder and upper arm. He needed two surgeries. At the time of trial, he was still taking narcotic medication for pain. The plaintiff suffered from serious limitation of movement of his shoulder and arm and from chronic pain. He was partially disabled and could not return to his previous work. He was working part-time as janitor and handyman. He was receiving Ontario disability pension for his permanent disability. He would never regain full motion or strength in his right shoulder and arm and would continue to suffer from his injuries in the future. He was determined to be 50% contributorily negligent and was awarded \$75,000 for general damages, \$103,357 in 2024 dollars. (\$150,000 if not contributorily negligent, \$251,943 in 2024 dollars)

- [169] In *MacKinnon v. Grand River Hospital*, 2007, ONSC 23492 after surgery, the plaintiff sustained injury to left arm and hand. She was in constant pain which left her whimpering almost incessantly. She lived an isolated life, having withdrawn from her family and children. She required a large number of medications and was unable to work again. She also experienced depression. She was awarded \$175,000 general damages, \$251,943 in 2024 dollars.

- [170] In *Miller v. Her Majesty the Queen*, 2015 ONSC 669, plaintiff sustained a left shoulder injury and the court found that he continued to experience constant and debilitating pain resulting in significant impairment of function and detrimental impact on his life including an undermining and prevention of his ability to carry on with recreational activities that gave him joy. He was awarded \$100,000 for general damages, \$126,750 in 2024 dollars.

- [171] In *Rizzi v. Marvos*, 2008 ONCA 172 the plaintiff sustained a leg injury that resulted in chronic pain and fibromyalgia that prevented her from continuing any employment. She was awarded \$41,000 for general damages at trial taking into account her 75% contributory negligence. On appeal, the court substituted a general damages assessment of \$80,000, reduced by 75% to \$20,000.

Cases regarding general damages presented by the defendant

- [172] In *Lawson v Costco Wholesale Canada Ltd* (2010 ONSC 360, para. 18), the 69-year-old plaintiff was injured in a slip and fall and developed a frozen shoulder following the incident. Unlike Ms. Bradley, the plaintiff's injuries were not limited to the shoulder as the plaintiff also suffered injuries to her leg and developed significant knee pain. The plaintiff had a prior history of medical problems including back pains to her lower back due to stenosis radiating to her right leg and hip. She had visited her doctor before the fall. Her condition had stabilized. The plaintiff was awarded \$30,000 for general damages, \$41,350 in 2024 dollars.

- [173] In *Tennant v Fariba* (2013 ONSC 1676, para. 44) the 49-year-old plaintiff was involved in a motor vehicle accident in 2006. At the time of the hearing in 2013, the plaintiff continued

to suffer from injuries to his right arm, right shoulder, low back and left knee. He had his own business and was able to return to work with reduced function. The court stated,

On all of the evidence presented in this case, Mr. Tennant has suffered a permanent serious impairment of an important bodily function. He has suffered and continues to suffer chronic pain in respect of his right shoulder, right arm, and hand associated with lack of strength, weakness and impairment of sensation. He continues to experience problems with this in respect of his lower back and pain in his left knee. He has received physiotherapy and acupuncture treatments which provided transient relief. He has taken medication in the form of pain killers which only provided temporary relief. His pain in the right shoulder, right arm and left knee was described as constant. He only obtains relief in respect of his pain complaints regarding his left knee when he sleeps. He wears a pad in respect of his left knee every day. He cannot work the way that he did before as a flooring installer in his own business. He now tends to rely more on subcontractors to do the heavy physical work that he could do before. He cannot do the heavy lifting and repetitive work involving his arms and legs that he did before this accident. He is right hand dominant and cannot use his right arm as he did before to install flooring material and he cannot use his left knee also as he did before. He cannot do the repetitive lifting, bending and squatting as he was able to do before the accident. He did not suffer from these types of problems before the accident but has experienced all of these difficulties since December 8, 2006.

[174] The plaintiff was awarded \$35,000 for general damages, \$45,729.93 in 2024 dollars.

[175] In *Jeremiah v Hamilton-Wentworth Police Services Board* (2018 ONSC 5762 para. 204) the plaintiff suffered injuries in the process of being falsely arrested. The plaintiff's main complaint was pain to both shoulders, radiating to the arm. X-rays and ultrasound showed supraspinatus and tears in both rotator cuffs. He was also diagnosed with PTSD and reported difficulty sleeping. His prognosis was guarded even with treatment. He was able to return to work as an auto mechanic but needed to rest 20% of the time while working. The plaintiff was awarded \$40,000 for general damages, \$48,197.46 in 2024 dollars.

[176] In *Radmanish v Sulaimankhail* (2014 ONSC 4373 para. 129), the 40-year-old plaintiff was physically assaulted. At trial, the plaintiff was still experiencing ongoing right shoulder pain, neck pain, anxiety, nightmares, flashbacks, and PTSD. The evidence established that these symptoms were likely to continue. The plaintiff was still experiencing ongoing right shoulder pain, neck pain. This prevented him from lifting things. He could not work as well as he did in the past but was able to return to work.

[177] The plaintiff was awarded \$39,900 for general damages, \$51,096.10 in 2024 dollars.

Assessment

[178] Dr. Auguste determined that Ms. Bradley's current impairments were likely permanent.

[179] She is no longer able to enjoy the activities that she did prior to the incident. She cannot go kayaking anymore. She tried once but had too much pain. She has not gone camping since the incident. She went for two short hikes this year. Her swimming now consists of floating and dog paddling. She has chronic pain which becomes worse when she does certain activities which are set out below.

[180] Taking all of the above into account, I award Ms. Bradley \$70,000 for general damages.

What are Ms. Bradley's future care needs?

Pre-incident Housekeeping

[181] Before the incident, Ms. Bradley and her husband lived in Courtice. Their house was a side split with three bedrooms upstairs, three bathrooms, one being on the main level and two upstairs and laundry on the main level. It was approximately 2500 to 2700 ft.² Ms. Bradley stated that she did all of the housekeeping except for outdoor maintenance which are husband did. She estimated that the division of labour was 90/10 with her doing 90% of the indoor work. Her husband cut the grass, shoveled the snow, vacuumed the pool and other jobs that she needed done in the house such as repairing something. She did vacuuming, the laundry, cooking, and cleaning the floors and windows. They opened and closed the pool together. She had no trouble doing the housekeeping up to December 1, 2015.

Pre-incident Personal Care

[182] Ms. Bradley stated that before the incident, she took great pride in her appearance and did not need any help with her personal care. She would shower every day, put on her makeup and get dressed. She had jewelry to complement her outfits, which she enjoyed wearing.

[183] Ms. Bradley stated that she does not wear a variety of jewelry now. She stated that she often wears her hair in a bun and looks awful. She does not see the sense of changing her accessories.

[184] Ms. Bradley and Grant Bradley testified about the split in housekeeping and home maintenance tasks in the home prior to December 1st, 2015. 408. Ms. Bradley testified about what she is able to do now considering her shoulder injury.

[185] Prior to December 1, 2015, Ms. Bradley was responsible for 90% of the housekeeping and home maintenance. Ms. Bradley and her husband lived in their 2,800 square foot home in Courtice, Ontario, which was a side split with three upstairs bedrooms. Ms. Bradley did most of the indoor housekeeping including the laundry, cooking, vacuuming and bathroom cleaning. She also completed some minor handyman tasks such as hanging pictures and

cleaning windows. Before December 1, 2015, both Ms. Bradley and her husband did all of the pool maintenance including opening and closing the pool. Grant took care of the outdoor tasks such as lawn maintenance, snow removal and moving of heavier things such as furniture for cleaning.

- [186] Ms. Bradley and Grant testified that in 2016 and 2017, Ms. Bradley was not able to do anything around the home. She rested on the couch. Grant took over 90% of the housekeeping. Ms. Bradley did a small amount of the work, depending on her pain level.
- [187] In 2019 and 2020, Ms. Bradley improved and was gradually able to do more housekeeping work. She started doing one or two tasks a day. For instance, she would do some floor swiffering, maybe some cooking and maybe some laundry, however, Grant had to carry the baskets to and from the laundry machines.
- [188] Ms. Bradley has not been able to return to her pre-incident ability to do housekeeping. Some of the housekeeping jobs are not done. The work that she does do can exacerbate her pain.

Experts

- [189] Two experts, Hazel Bowles and Angela Flemming, testified regarding Ms. Bradley's needs and the cost of them. Hazel Bowles was qualified as an expert occupational therapist and life care planner permitted to give opinion evidence on the medical care, personal care, attendant care and housekeeping and home maintenance needs arising from Ms. Bradley's functional deficits and impairments, and to provide costing of these needs. Angela Fleming was qualified as an expert in occupational therapy, rehabilitation and life care planning and was permitted to give opinion evidence on Ms. Bradley's future care needs and to provide a costing of those needs. Ms. Fleming was retained to review Ms. Bowles' report dated June 7, 2022. She was also retained to provide an independent analysis of Ms. Bradley's future needs and associated costs.

Hazel Bowles' assessment

- [190] Ms. Bowles stated that her mandate was to consider Ms. Bradley's circumstances prior to the event and what it would take to get her back there. She considered Ms. Bradley's impairments in disability, the course of her impairment over time, her social demographics and familial factors. In order to do this, she reviewed Ms. Bradley's medical documents, which are listed in her report, that were provided by her counsel in order to understand what her impairments might be. She interviewed Ms. Bradley in her home to get an understanding of her disabilities. She toured the home and looked at areas that were providing Ms. Bradley with challenges.
- [191] Ms. Bowles carried out an in-person assessment of Ms. Bradley on May 8, 2018. She did not do a physical assessment, rather, she did a brief functional range of motion assessment. Ms. Bowles stated that at that time, Ms. Bradley was 53 years old, and was right-hand dominant. Ms. Bowles estimated that Ms. Bradley had three quarters of full range of motion. She reported that she had learned to live with her limitations. She had pain from

her left shoulder radiating to her upper back and neck. She was doing daily exercises to achieve a better range of motion. She was using a TENS machine for flareups.

- [192] Ms. Bradley lived in Marmora in a large century home. She had a lot of difficulty with her upper left extremity - she could not put a dish in her upper kitchen cabinet. Because she was not working, the family income decreased. Ms. Bowles noted that the Marmora house had many extensions, upgrades and renovations. It had an unusual layout.
- [193] Ms. Bowles stated that Ms. Bradley is a vegan. Her food preparation involved more chopping which was difficult for her to do. She was relying on prepared food and take out. She had gained some weight. Her husband had to take over some of the cooking because she could not consistently do it.
- [194] Regarding the management of her symptoms, Ms. Bradley told Ms. Bowles that she had tried different pain medications but that they caused her nausea. She was taking just regular Tylenol. She had tried massage, chiropractic, acupuncture and physiotherapy. She stated that physiotherapy had been the most helpful. She had consistent treatments until 2017 when she concluded that it was not helping anymore. In addition, because of the move, she was not near her physiotherapist. She was doing exercises at home and was not seeing anyone consistently.
- [195] Ms. Bowles stated that Ms. Bradley told her that before the incident, she took pride in her appearance. She had long hair. She wore makeup and jewelry. She wore high heels when socializing. After the incident, she cut her hair shorter so that it would be easier to manage. She started showering only 2 to 3 times per week because managing her hair was a burden. She stopped drying her hair because she could not hold a blow dryer and brush over her head. She just wore it up in a clip. She gave up wearing high heels. She had difficulty putting on her jewelry.
- [196] After the incident, Ms. Bradley could not do much meal preparation. Her husband was doing it. He was helping her with heavier lifting, getting plates out of the cupboards. He was doing the vacuuming and floor care. She could not clean blinds. She could do some things slowly with her right arm but only one task a day or her left shoulder would flare up. Her husband would assist with regular tasks such as carrying laundry baskets.
- [197] Prior to the incident, Ms. Bradley and her husband did all the work to open their swimming pool. After the incident, they hired people to do this because she could not assist and her husband was doing many other things.
- [198] Ms. Bowles stated that Ms. Bradley reported that she was driving less. Destinations beyond 45 km away were beyond her pain tolerance.
- [199] Ms. Bowles stated that Ms. Bradley's reported impairments were consistent with her examination. Ms. Bowles prepared a report dated June 1, 2018 in which she identified Ms. Bradley's needs and provided a costing for them. She stated that her company has an extensive provincewide database which is specific for treatment. Ms. Bowles stated that

she assumed that Ms. Bradley would not return to work, she would remain in her current house, and would not regain full function of her left upper extremity.

- [200] Ms. Bowles stated that she considered assistive devices. She also considered professional services consistent with how Ms. Bradley had managed up to the current date and what she might need going forward. While the cost of medical specialists would be covered by OHIP, there would be travel costs associated with appointments. Ms. Bowles stated that she considered the therapies that Ms. Bradley had attempted in the past including physiotherapy massage chiropractic and acupuncture. She opined that Ms. Bradley would try different kinds of therapies depending on her needs. Ms. Bowles stated that she combined them together and averaged them out into a reasonable number of sessions.
- [201] Ms. Bowles stated that Ms. Bradley had reported guilt, frustration and sadness after the incident. She had not seen a counsellor. Ms. Bowles recommended a fund that could be used for counselling if she needed it. Ms. Bowles stated that she recommended an occupational therapist to assist Ms. Bradley in exploring leisure, social and recreational activities that she could do during the day within her limitations.
- [202] Ms. Bowles noted that because Ms. Bradley and her husband had reduced their vehicles down to one car, she would need taxis for any medical appointments so as to not be limited by her husband's schedule.
- [203] Ms. Bowles stated that Ms. Bradley had developed a number of compensatory methods for doing certain things and that she relied on her husband. He had taken over meal preparation and was providing a service to her. Ms. Bowles stated that this cannot be relied upon for lifelong services. A principle of life care planning is that one cannot rely on family and friends for assistance. Ms. Bowles considered Ms. Bradley on her own, quantified the hours that her husband was spending to assist her. She used that to calculate the cost of a personal support worker.

Angela Flemming's assessment

- [204] Ms. Flemming also reviewed Ms. Bradley's medical and rehabilitation records, which are listed in her report, that were provided by her counsel in order to understand what her impairments might be.
- [205] Ms. Flemming conducted an occupational therapy assessment. She observed Ms. Bradley in a variety of positions and doing a number of activities over two hours. She noted Ms. Bradley's pain and fatigue levels. In her analysis, she noted that Ms. Bradley relies primarily upon her right upper extremity and was able to incorporate her left upper extremity in a more supportive fashion. Using this approach, with periods of rest, she had limited pain and fatigue. Her left shoulder symptoms impact on specific tasks. She avoids heavy carrying, hanging heavy items on the outside clothes line and folding them, cleaning windows, pool openings and closings, moving furniture for seasonal cleaning, vacuuming the stairs, carving and sculpting and kayaking. To her credit, Ms. Bradley has learned to accommodate for her left shoulder for the vast majority of her day-to-day activities.

TENS machine, electrodes and leads

- [206] Ms. Bowles stated that Ms. Bradley used a TENS machine and would need several of them over the span of her lifetime. She used a value of \$176.64, multiplied it by three and arrived at a one time cost of \$529.92. She used a value of \$48.48 for the electrodes and leads. She multiplied this by 4 and arrived at an annual recurring cost of \$193.32.
- [207] Ms. Flemming recommended \$200 per year for symptom management which could be used for replacement of TENS electrodes and other modalities such as acetaminophen.
- [208] I accept Ms. Bowles' evidence that Ms. Bradley should have sufficient funds to replace her TENS machine as required along with the electrodes. Ms. Flemming's amount does not cover a one time purchase of a TENS machine and one set of electrodes.

Heat and Ice Packs

- [209] Ms. Bowles stated that Ms. Bradley would benefit from heat and ice packs valued at \$2.50 with an annual recurring cost of \$7.32 to be replaced every 2 to 3 years.
- [210] I do not accept that they are required. Ms. Bradley has not been using heat and ice packs.

Assistive Devices

- [211] Ms. Bowles stated that assistive devices such as a reacher, a blow dryer stand, a bath sponge and Swiffer duster would assist Ms. Bradley. There would be a single \$155 purchase cost. In addition, \$38 per year would have to be spent to replace these items from time to time resulting in an annual recurring cost of \$38.
- [212] Ms. Flemming stated that Ms. Bradley could not identify any additional assistive devices that she felt would be beneficial regarding self-management of her symptoms.
- [213] I accept that these devices would assist Ms. Bradley with her activities of daily living and self-care.

Pain Medication

- [214] Ms. Bradley uses Tylenol ES to manage her symptoms but not every day. Ms. Bowles stated that allowing for 2 pills weekly would be appropriate.
- [215] Ms. Flemming stated that Ms. Bradley was not taking any medication.
- [216] I accept that there should be an allowance for medication so that Ms. Bradley can take it when she needs it.
- [217] Ms. Bowles costing is 0.14 weekly: annual recurring cost \$14.56

Travel to Physician Visits

- [218] Ms. Bowles determined that Ms. Bradley would incur mileage costs to visit her family doctor and Dr. Mason.
- [219] I do not accept that a cost for mileage is required. Ms. Bradley has not seen Dr. Mason since 2019. She would incur mileage costs to see her family doctor for other unrelated issues.

Pain Management Services

- [220] Ms. Bowles stated that Ms. Bradley had tried several therapies such as physiotherapy, chiropractic treatment, acupuncture and massage. She stated that the average cost for services of \$74.28 per treatment in Peterborough for 8-16 treatments per year. The annual recurring cost is \$891.36.
- [221] Ms. Flemming stated that Ms. Bradley has bi-weekly massage therapy which is more than would be expected given her functional status and ongoing improvement. She goes to a clinic in Madoc which charges \$60 for 30 minutes. She recommended \$60 up to 26 x per year totaling \$1,560 per year for life for symptom management. This is 30 minutes of treatment every second week.
- [222] Both experts agree that an amount for treatment is required. Ms. Bradley testified that she was obtaining massage therapy. I accept Ms. Flemming's recommendation because it is based on the clinic price that Ms. Bradley attends.

Travel to Pain Management Services

- [223] Ms. Bowles based her calculations on travel to Peterborough or Belleville. Ms. Bowles stated that the distance from Ms. Bradley's home to Madoc and back is 34 km. The annual recurring cost would be \$230.
- [224] I accept that travel to pain management services should be based on the distance to Madoc

Psychological Counselling

- [225] Ms. Bowles recommended an amount for counselling and mileage. I do not accept that this is necessary because Ms. Bradley has not had any psychological treatment.

Occupational Therapy

- [226] Ms. Bowles recommended an amount for occupational therapy and mileage. I do not accept that this is necessary because Ms. Bradley has not had any occupational therapy.

Personal Care Services

- [227] Ms. Bradley stated that she no longer takes a shower each day because the pain in her left shoulder makes washing her hair difficult. She no longer styles her hair the way she did prior to the incident. She cannot hold a blow dryer and a brush over her head. She does not wear make-up nor does she change her jewellery. Her personal grooming has been reduced because of her pain. Ms. Bowles states that some personal care services would assist her.
- [228] Ms. Flemming states that Ms. Bradley has learned to adapt. She does not need any personal care services.
- [229] Ms. Bradley has learned to adapt; however, she has not regained her pre-incident abilities. I accept that some personal care services would assist her.
- [230] Ms. Bowles' costing for personal care services at 40.28 per hour, 234 hours per year results in an annual recurring cost \$9,425.52.

Housekeeping

- [231] Ms. Bowles stated that in future care planning, using a decreased amount due to reliance on a spouse or relative's gratuitous work is an error. Relatives' circumstances may change. They cannot be counted upon to provide services. Ms. Flemming stated that taking into account Grant's help is reasonable, especially because he is retired. I accept Ms. Bowles evidence. In a perfect world, Grant would be available and fit; however, if he were not, Ms. Bradley would have difficulty managing. Four hours of weekly housekeeping services are reasonable.
- [232] Ms. Bowles' costing at \$48.73 per hour for 4 hours weekly including HST, \$208 per year results in an annual recurring cost \$10,135.84.

Home maintenance/handyman services

- [233] Ms. Bradley lives in a century home. Some handyman services up to when Ms. Bradley is 77 years old are reasonable, considering her limitations and the fact that reliance on Grant cannot be taken into account. I accept that 15 hours per year should be provided.
- [234] Ms. Bowles costing at \$55.77 for 15 hours per year to age 77 results in an annual recurring cost \$836.55.

Summary

<u>Item</u>	<u>Time</u>	<u>Annual Cost</u>
TENS machine	fixed	529.92
TENS electrodes and Leads	annual	193.92
assistive devices	fixed	155
assistive devices	annual	38
Tylenol ES	annual	14.56
Pain management services	life	1,560

Travel to pain management services	life	230
Personal care services	life	9,425.52
Housekeeping	life	10,135.84
Handyman services	age 78	836.55

Evidence of Stephanie Greenwald

- [235] Stephanie Greenwald was qualified as an expert accountant to give opinion evidence with respect to the costs of Ms. Bradley's future care.
- [236] Ms. Greenwald assumed that Ms. Bradley would have a life expectancy equal to the general female population in Canada, being 86.28 years. In accordance with r. 53.09(1) of the *Rules of Civil Procedure*, she noted a 0% discount rate for the first 15 years and then 2.5% thereafter. She also assumed that future professional services will increase at a rate above inflation and that a discount rate of 1.0% per annum, in accordance with r. 53.09(1) would apply. She also suggested a financial management fee. I do not accept that a management fee is necessary in these circumstances.

Evidence of Professor Hyatt

- [237] Professor Hyatt provided some calculations regarding Ms. Bradley's on-going needs; however, they are based on Ms. Flemming's evidence which I largely reject.
- [238] The plaintiff is requested to provide a calculation on the cost of Ms. Bradley's future care, based on the amounts in the above-noted chart. If the parties do not agree on the calculations, the defendant may provide a responding calculation.

What is Ms. Bradley's Loss of Income?

When could she have returned to work and in what capacity?

- [239] Ms. Bradley did not return to work after December 9, 2015. She stated that she discussed it with her doctors who determined that she could not carry on with her regular ED nurse work. She was not cleared for return to work. Dr. Brankston filled out forms for CCP disability benefits which she received.
- [240] There is conflicting evidence about whether Ms. Bradley could have returned to work in 2016. Ms. Bradley's physicians stated that she could return to sedentary work in 2019, four years after the incident.
- [241] As noted above, between December 2015 and August 2021, Dr. Brankston and Dr. Bathe, filled out 21 forms regarding Ms. Bradley's disabilities for various entities including Employment Insurance Sick Benefits, Lakeridge Hospital, Bank of Montreal, Sunlife Financial, Johnson Inc. LT, Health Care of Ontario Pension Plan (HOOPP) and Manulife. With the exception of January 26 and February 25, 2016, up until June 2020, the forms stated that Ms. Bradley could not work as an ED nurse or in any occupation. On June 5,

2020, Dr Bathe completed a form for HOOPP which stated that Ms. Bradley could not work at her own occupation but was able to work at any occupation full time or part time. Dr. Bathe stated that at this point, Ms. Bradley could work at jobs such as a Walmart greeter or a job with voice activation.

Dr. Smith's opinion

- [242] Dr. Smith first saw Ms. Bradley on June 8, 2022, five and a half years after the incident. He stated that Ms. Bradley could have returned to work with modified duties in 2016. He seems to have based this on the two PRM reports that Dr. Brankston completed in January and February 2016; however, from then up to his retirement in September 2016, Dr. Brankston stated on various forms that she was unable to return to work. For the remainder of the year, Dr. Bathe completed forms indicating that Ms. Bradley was unable to return to work.

Dr. Ruggles' opinion

- [243] Dr. Ruggles, who first saw Ms. Bradley on January 22, 2019, four years after the incident, stated that he could not imagine her returning to work in 2016. Her condition was worse and she was still having pain three years later in 2019. He stated that she would not be able to return to ED nursing, given the physical demands. He commented that she could possibly work in a lighter nursing position.
- [244] On June 25, 2020 Dr. Bathe completed a form stating that Ms. Bradley was able to work part-time. He stated that her physical limitation prevented her from working as an ER nurse. By then, Ms. Bradley was 55 years old, four and a half years post incident and had an on-going disability.
- [245] As noted above, on August 14, 2021, Dr. Auguste, an orthopedic surgeon, carried out an assessment of Ms. Bradley to determine whether she was totally disabled as defined by HOOPP. Dr. Auguste determined that Ms. Bradley's current impairments were likely permanent. She was not disabled from any work as a RN. She was unable to return to her position as a RN in Lakeridge Health's ED. Dr. Auguste recommended a job change to a "more administrative, less physical and more supervisory related position in the field of nursing".

Analysis

- [246] Dr. Brankston's notations in January and February 2016 that Ms. Bradley could return to work appear to be outliers. For the remainder of 2016, both he and Dr. Bathe stated on various forms that she could not return to work. I give less weight to Dr. Smith's opinion. He saw Ms. Bradley five and a half years after the incident. I give more weight to her treating doctors because they saw her almost monthly in 2016 and were in the best position to determine whether she was capable of returning to her job then.
- [247] Dr. Ruggles, stated in January 2019 that Ms. Bradley could return to a light duty nursing job. Dr. Bathe saw Ms. Bradley on March 25, 2019 and May 4, 2020; however, he did not

address a return to work then. In his report dated June 25, 2020, Dr. Bathe stated that Ms. Bradley could work part time. I accept Dr. Ruggles' evidence that Ms. Bradley could return to light duty nursing in January 2019. At this time, Ms. Bradley was four years post event and was 54 years old.

What type of job would have been available to Ms. Bradley?

Evidence of Christie MacGregor

- [248] Ms. McGregor was qualified as an expert in vocational rehabilitation and labour market qualified to opine on vocational opportunities available to Ms. Bradley as well as the process for returning workers like Ms. Bradley after an injury.
- [249] Ms. Bradley stated that hospitals have a duty to accommodate employees and assist them to return to work. For example, Ms. McGregor stated that Ms. Bradley had a workplace injury in 2010 after which she was accommodated in her return to work. Ms. Bradley was a member of the Ontario Nurses of Ontario collective agreement which also has return to work provisions. Ms. Bradley did not engage in any return to work plan.
- [250] Ms. MacGregor stated that Ms. Bradley has a number of transferrable skills. Nursing is one of the most in demand occupations now. Lakeridge Health would have been able to find a job for Ms. Bradley that accommodated her limitations. In 2023 when she did the research for her report, Ms. MacGregor found two postings, a unit manager and a mental health and addiction position. If Lakeridge did not have a suitable position, there were other positions outside of the hospital. Ms. MacGregor created a chart of positions that she believed would be more sedentary and suitable for Ms. Bradley's restrictions.

Analysis

- [251] The difficulty with the positions set out in Ms. MacGregor's chart is that all of them would require a long commute. The closest location is Toronto. Others are considerably farther. Ms. MacGregor stated that the chart was meant to show the types of positions generally available, not necessarily that they would be manageable for Ms. Bradley. This type of evidence does not assist the court. Ms. MacGregor agreed that most nursing positions would require CPR skills. This involves putting one's body weight into pressing on to a person's chest to do compressions. Dr. Smith stated that Ms. Bradley could not pull or push more than 10 lbs. While there was no evidence of whether Ms. Bradley could perform CPR, I find that it is unlikely.
- [252] The incident occurred at the end of 2015. Ms. Bradley and her husband moved to Marmora in 2017, which was not unreasonable. The evidence shows that Ms. Bradley could not have returned to work before 2019.
- [253] The location of Ms. Bradley's residence must be taken into account in determining suitable employment for her. Her evidence was that she could drive 45km. More causes her pain. The defendant did not provide any evidence of a suitable job for Ms. Bradley within a reasonable commute. In 2019, Ms. Bradley had not worked for four years, was 54 years

old and had a disability. I accept the plaintiff's submissions that employers tend to avoid prospective employees with physical limitations. (see *Baron v. Nguyen*, 1992 O.J. No. 609 para. 49 People over 45 years of age have more difficulty finding work than younger people. (see *Williams v. Air Canada*, 2022 ONSC 6616, para. 16) Ms. Bradley would be at a competitive disadvantage in applying for a job in contrast to a younger, fit person who had been steadily employed. Furthermore, when Covid struck in 2020, many employees could not work.

- [254] While Ms. Bradley had the physical ability in 2019 to work at a sedentary job part time, I conclude that finding and obtaining a suitable position would have been almost impossible for her.

Past Loss of Income

Evidence of Stephanie Greenwald

- [255] Stephanie Greenwald was an expert accountant qualified to give opinion evidence with respect to Ms. Bradley's past and future income loss and pension loss. Based on Ms. Bradley's continuing to work as a nurse, Ms. Greenwald calculated the following:

past loss of income from December 2, 2015 to September 27, 2022	\$754,911
present value of Ms. Bradley's ongoing without incident income from September 28, 2022 if she retired at age 60	\$291,955

- [256] At age 60, Ms. Bradley would be eligible for an unreduced pension. Ms. Bradley testified that her plan was to retire at age 60. Ms. Greenwald used Ms. Bradley's projected 2022 income of \$117,267, then considered how many years she would have to live to age 60 and then applied the 2.49 discount factor which resulted in \$291,195.

Evidence of Professor Douglas Hyatt

- [257] Professor Hyatt was qualified as an expert in economic loss to give opinion evidence with respect to Bonnie Bradley's past and future income loss and pension loss as well as the calculation of future costs of care. Professor Hyatt noted that Ms. Greenwood's calculations assumed that Ms. Bradley would not return to work. He stated that she failed to deduct the CPP disability benefits that Ms. Bradley received. In 2016 and 2017, she reported receiving \$9,012 and \$12,185. He assumed that the benefit had increased from 2017 by the rate prescribed by Canada Pension Plan administrators as follows:

2016	9,012
2017	12,185
2018	12,368
2019	12,652
2020	12,892
2021	13,021

2022	9,892 (up to September 27, 2022)
Total	\$82,022

- [258] Professor Hyatt stated that in 2017 Ms. Bradley reported T4 employment income of \$3,824 which he assumed included in part Ontario Nurses Association disability benefits of \$250 per month. He then assumed that she would receive an annual ONA disability benefit of \$3,000 per year (\$250 x 12 months). He calculated that Ms. Bradley would have received \$18,043 from 2017 to 2022.
- [259] There is no evidence that Ms. Bradley received an ONA disability benefit. Professor Hyatt made assumptions. Therefore, I cannot conclude that she received \$18,043.
- [260] Professor Hyatt calculated the present value of Ms. Bradley's CPP disability benefits using an annual amount of \$13,373 x 2.49 to be \$33,299. I find that deducting the CPP benefit is reasonable in calculating Ms. Bradley's past and future loss of earnings.
- [261] Using Ms. Greenwood's calculation of \$754,911 for past loss of earnings and subtracting \$82,022 (the total CPP benefits) Ms. Bradley's past loss of earnings is \$672,889.
- [262] Using Ms. Greenwood's calculation of \$291,995 for her future loss of earnings and subtracting the present value of the CPP benefits, \$33,299, Ms. Bradley's future loss of earnings is \$258,696.
- [263] Accordingly, Ms. Bradley's loss of income is \$931,585.

What is Ms. Bradley's pension loss?

Evidence of Stephanie Greenwald

Ms. Greenwald calculated Ms. Bradley's pension loss as follows:

projected pension income absent incident

projected annual pension from age 60 to age 65 (includes bridge amount)	\$44,439
multiplier	4.89
present value of pension loss age 60 to 65	\$217,307

Projected annual pension from age 65	\$38,892
Multiplier	18.42
Present value of projected pension income from age 65 to life	\$716,388
present value of projected pension income absent the accident as at September 28, 2022	\$933,695

Projected post-accident pension income

Projected annual pension from age 60 to age 65	\$23,767
Multiplier	4.89
Present value of projected pension income from age 60 to age 65	\$116,221

projected annual pension from age 65	\$20,200
multiplier	18.42
present value of projected pension income from age 65 to life	\$372,084
present value of projected pension income post accident as at September 28, 2022	\$488,305

Pension contribution savings

value of past contribution savings to September 28, 2022	\$60,442
present value of future contribution savings from September 28, 2022 to retirement	\$23,348
Value of past and future contribution savings as at September 28, 2022	\$83,790
Present value of future pension losses as at September 29, 2022	\$361,600

Evidence of Professor Hyatt

[264] Professor Hyatt's only comment on Ms. Greenwald's pension loss calculation is that it assumes Ms. Bradley is unable to return to work. If Ms. Bradley had an employer sponsored pension plan, then the future pension losses would be less than Ms. Greenwald's calculation.

[265] There is no evidence that Ms. Bradley had an employer sponsored pension plan. I accept Ms. Greenwald's calculations.

What are Ms. Bradley's out of pocket expenses?

[266] Ms. Bradley provided a list of her out of pocket expenses from March 14, 2016 to September 29, 2016 totalling \$14,418.26. They consist primarily of parking costs and massage treatments; however, there are two entries for tax payments for RRSP: \$11,333.09 and \$421.78. There is also an entry for HOOPP pension top up of \$1,463.21. These amounts comprise 90% of the expenses. Ms. Bradley did not provide any testimony regarding the list of expenses. I cannot determine what if any liability the defendant has for the tax payments and pension top up. Accordingly, they are not allowed. I find that Ms. Bradley's out of pocket expenses are \$1,400.18.

What is OHIP's subrogated interest?

[267] The plaintiff advances an OHIP payment summary of \$2,493.24. A breakdown was provided. It includes amounts paid to Dr. Brankston and Dr. Mason as well as fees for X-rays and an MRI, all between December 14, 2015 and July 23, 2019. The defendant states that Ms. Bradley would have required medical attention for her frequent work-related injuries equivalent to the amount claimed by OHIP. Therefore, there is no basis for an OHIP subrogated claim.

[268] The X-rays and MRI were investigations related to Ms. Bradley's pain and shoulder symptoms. From the medical record before me, it seems that between 2012 and 2015, (not including the six attendances after the incident) Ms. Bradley saw her doctor 13 times for work-related pain. The OHIP summary shows that in 2016, Ms. Bradley saw a combination of Dr. Brankston and Dr. Mason 16 times. In 2017, Ms. Bradley saw Dr. Mason three times. In 2018 she saw him twice. In 2019 she saw him twice. But for the incident, Ms. Bradley would probably not have needed to see a physiatrist. From 2016 to 2023, she saw her doctors much more often than she did prior to the incident. The OHIP subrogated claim is allowed.

Summary

General damages	70,000
Cost of future care	to be determined
Past and future loss of income	931,585
Pension loss	361,600
Out of pocket expenses	1,400.18
OHIP subrogated interest	2,493.24

[269] If I have made any mathematical errors or omissions, counsel may contact my judicial assistant for directions.

Costs

[270] After the opinions on cost of future care have been provided, if the parties are unable to agree on costs, they may file written submissions, no more than 10 pages of text (14 pt. font size, regular 1" margins, 1.5 spacing) exclusive of a costs outline and any offers to settle. If either party exceeds the 10 page limit, the additional pages will not be considered. The plaintiff's submissions are due within four weeks. The defendant's submissions are due within a further three weeks. Either party may request an extension if necessary.

[271] The costs outline shall identify all lawyers on the file, their respective years of call and rates actually charged to the client. It shall also provide calculations on a partial indemnity basis including the rate applied, and if sought, the same on a full indemnity basis. A breakdown of the hours spent on specific aspects of the work shall be provided. A breakdown of disbursements shall be provided with copies of receipts if applicable. If the parties intend to rely on caselaw, hyperlinks shall be provided.

[272] In the costs submissions, the parties may also make submissions regarding the applicable pre-judgment and post-judgment interest rates on pecuniary and non-pecuniary damages.

[273] All costs submissions shall be provided to my judicial assistant by email. If no submissions are received after the defendant's submissions are due, costs will be deemed to have been settled between the parties. If the defendant does not provide costs submissions, the court will infer that the defendant does not contest the plaintiff's request for costs.

Justice M.E. Vallee

Released: April 29, 2025